

# **BASIC PRINCIPLES OF MEDICINE 1: FOUNDATIONS TO MEDICINE**

## **LECTURE FTM 45 Karyotypes and Aneuploidy**

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# Pre-Lecture - Required Reading and Objectives

- **Robbins Essential Pathology**, Chapter 6, section on Cytogenetic Disorders involving Autosomes and Sex Chromosomes:
  - [Genetic Diseases - Robbins Essential Pathology - ClinicalKey Student](#)

SOM.MKIII.BPM1.1.FTM.3.GNET.0121

Analyze the abnormal human karyotype and define the terms euploidy and aneuploidy. Use disorders and syndromes such as Down, Patau, Edward, Turner, Klinefelter as examples

# Recommended Reading and Objectives

- **Human Genetics and Genomics, Korf and Irons, 4<sup>th</sup> Edition Chapter 6**
  - **You can find the following pages and chapters on your Sakia site under:**
  - **Resources/General Information/Book Access**

|                                  |                                                           |
|----------------------------------|-----------------------------------------------------------|
| SOM.MKIII.BPM1.1.FTM.3.GNET.0120 | Analyze the normal human karyotype and how it is obtained |
|----------------------------------|-----------------------------------------------------------|

# Objectives

|                                  |                                                                                                                                                                             |
|----------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| SOM.MKIII.BPM1.1.FTM.3.GNET.0120 | Analyze the normal human karyotype and how it is obtained                                                                                                                   |
| SOM.MKIII.BPM1.1.FTM.3.GNET.0121 | Analyze the abnormal human karyotype and define the terms euploidy and aneuploidy. Use disorders and syndromes such as Down, Patau, Edward, Turner, Klinefelter as examples |
| SOM.MKIII.BPM1.1.FTM.3.GNET.0122 | Analyze meiosis to explain how chromosomal aberrations are formed                                                                                                           |
| SOM.MKIII.BPM1.1.FTM.3.GNET.0123 | Discuss mechanisms that might lead to mosaicism for chromosomal aneuploid conditions                                                                                        |

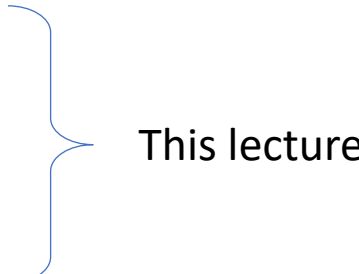
# Methods of Chromosome Analysis

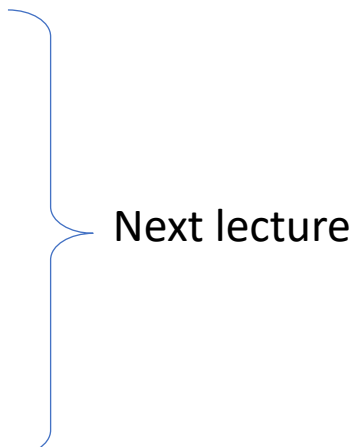
Laboratory techniques used for karyotyping

**Table 3-1. Development of Methodologies for Cytogenetics**

| Decade                                                                                                                                                                                  | Development                                  | Examples of Application                                                                                                                                           |
|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1950-1960s                                                                                                                                                                              | Reliable methods for chromosome preparations | Chromosome number determined to be 46 (1956) and Philadelphia chromosome identified as t(9;22) (1960)                                                             |
| 1970s                                                                                                                                                                                   | Giemsa chromosome banding                    | Mapping of <i>RB1</i> gene to chromosome 13q14 by identification of deleted chromosomal region in patients with retinoblastoma (1976)                             |
| 1980s                                                                                                                                                                                   | Fluorescent in-situ hybridization (FISH)     | Interphase FISH for rapid detection of Down syndrome (1994)<br>Spectral karyotyping for whole genome chromosome analysis (1996)                                   |
| 1990s                                                                                                                                                                                   | Comparative genomic hybridization (CGH)      | Mapping genomic imbalances in solid tumors (1992)                                                                                                                 |
| 2000s                                                                                                                                                                                   | Array CGH                                    | Analysis of constitutional rearrangements; e.g., identification of ~5 Mb deletion in a patient with CHARGE syndrome that led to identification of the gene (2004) |
| CHARGE, coloboma of the eye, heart defects, atresia of the choanae, retardation of growth and/or development, genital and/or urinary abnormalities, and ear abnormalities and deafness. |                                              |                                                                                                                                                                   |

# Chromosomal abnormalities detectable by cytogenetics

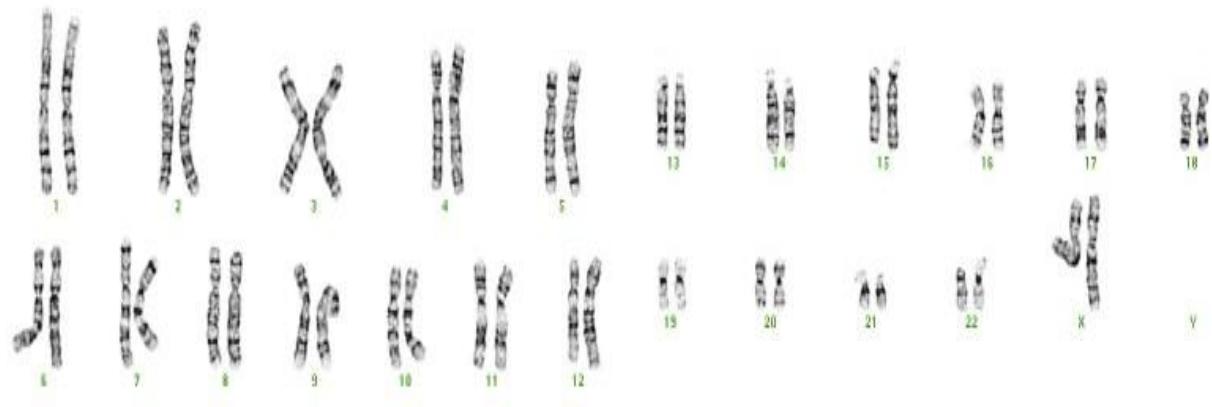
- Chromosomal abnormalities account for a large number of genetic malformation syndromes, spontaneous abortions, and infertility cases
- **Numerical Chromosomal abnormalities may be a result of:**
  - Non-disjunction during meiosis
  - Non-disjunction during mitosis
  - Mosaicism

This lecture
- **Structural Chromosomal abnormalities may arise from:**
  - Deletions
  - Duplication
  - Inversions
  - Translocations
  - Isochromosomes
  - Ring Chromosomes

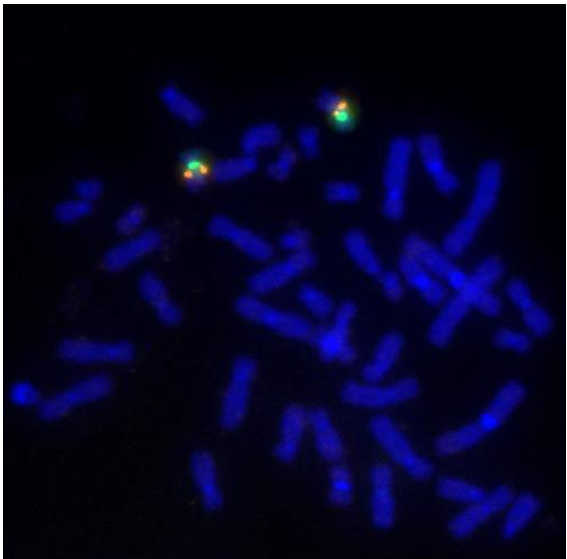
Next lecture

# Cytogenetic Testing

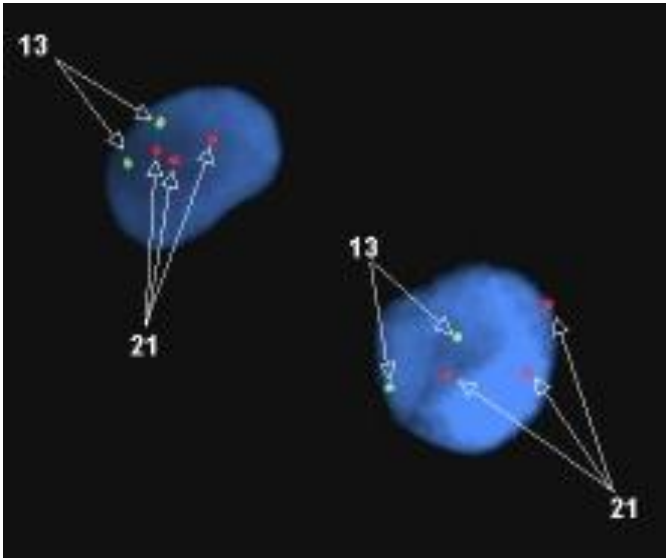
Karyotype, giemsa staining



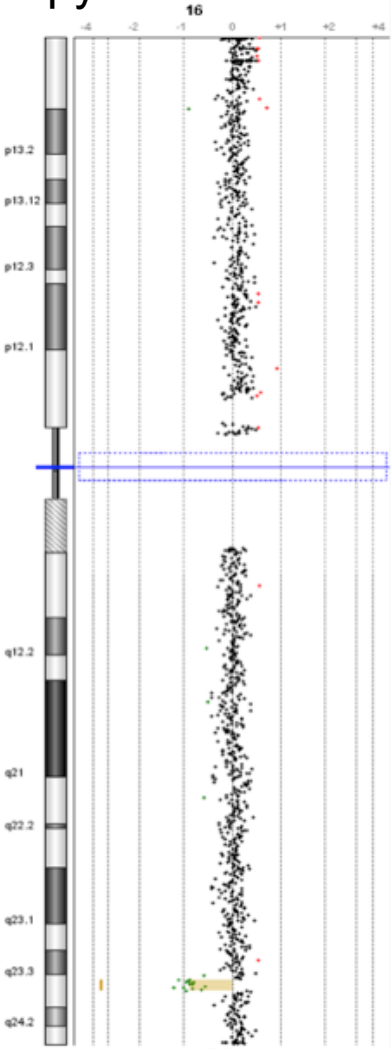
FISH fluorescent probes



Interphase FISH fluorescent probes



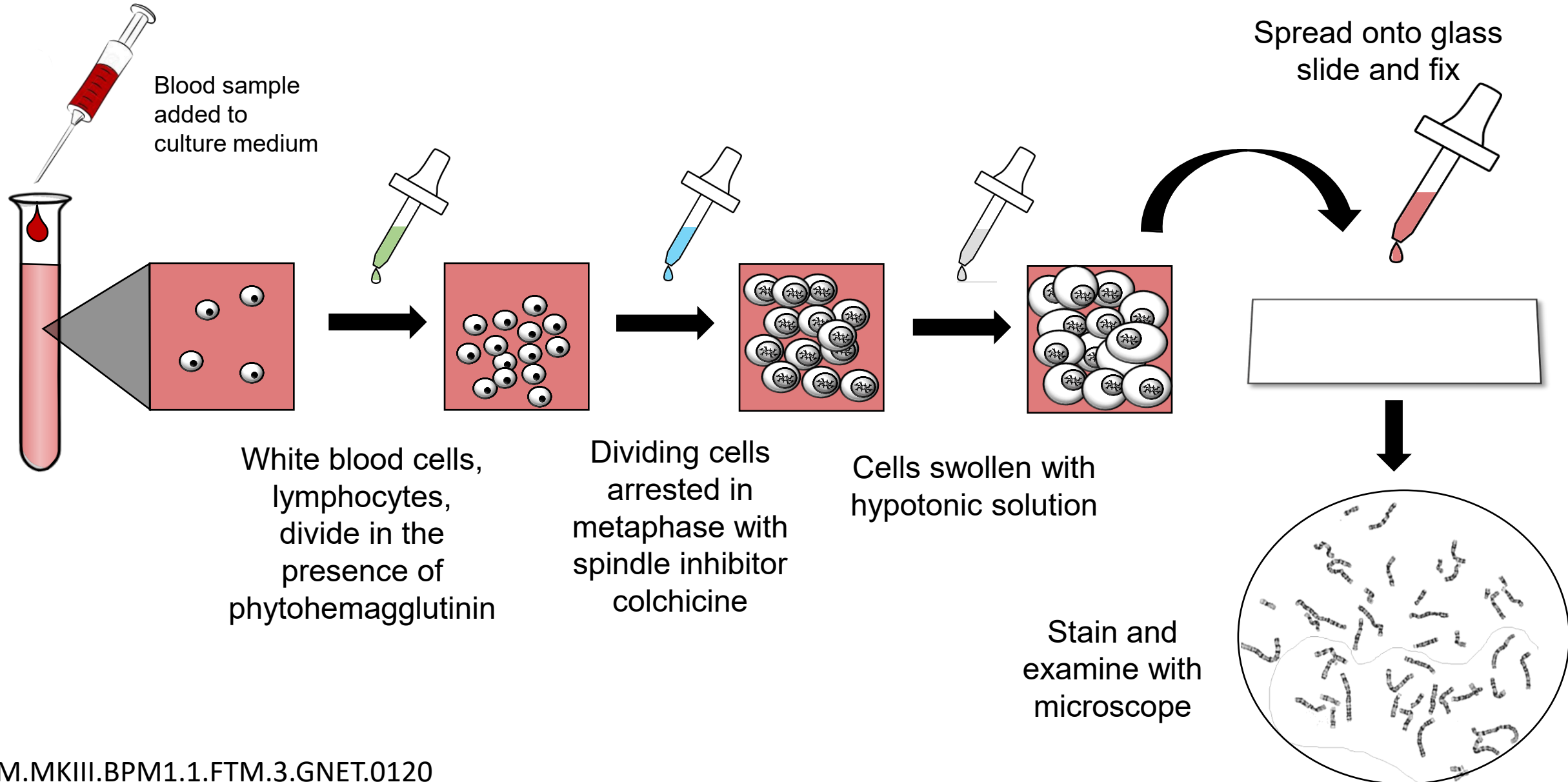
CGH Microarray  
Copy Number Variants



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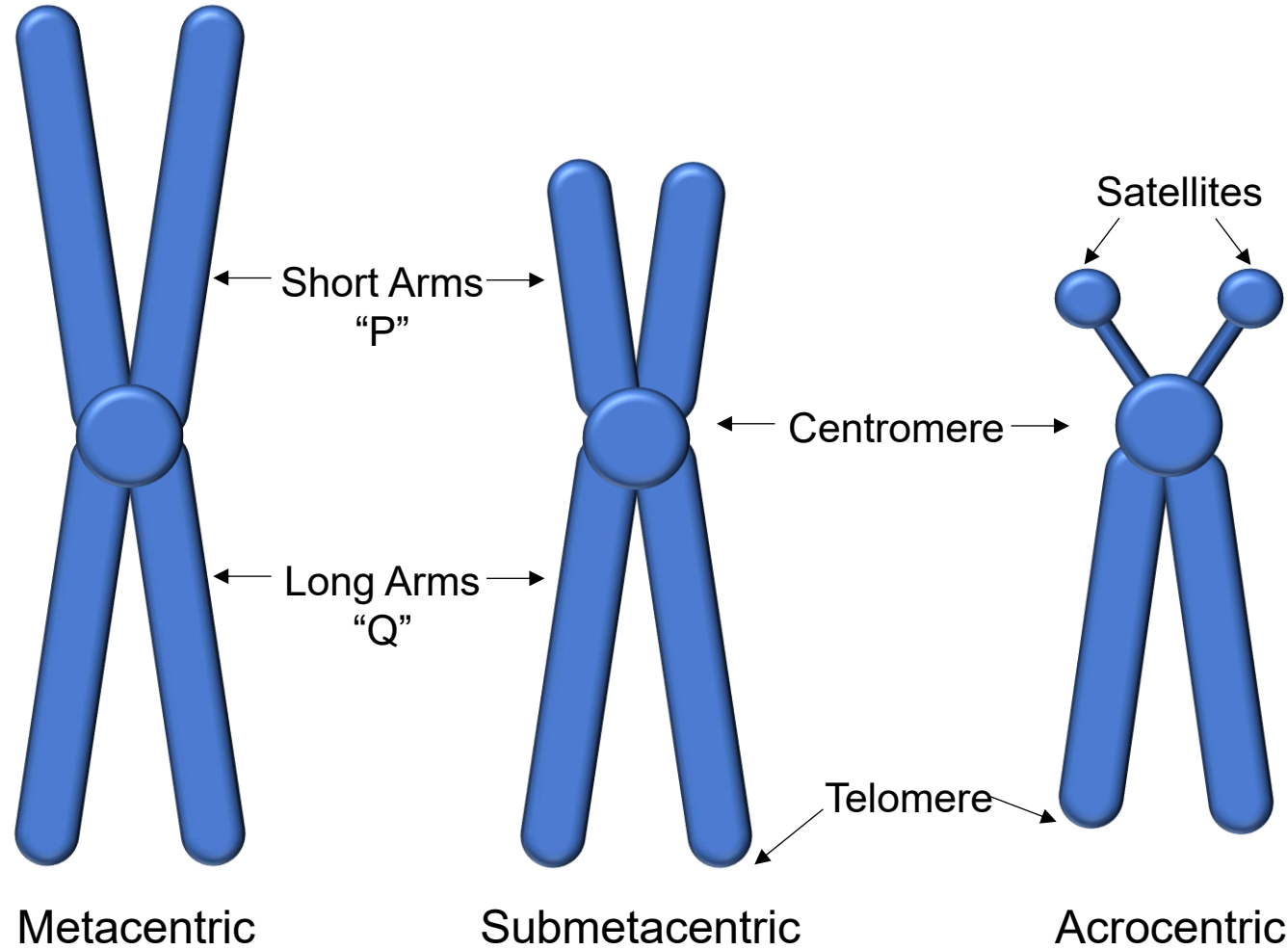
# Cytogenetics: Generating a karyotype



# Karyotype

- Karyotype is analyzed by microscopy
  - Only relatively large alterations will be observable by karyotype
  - Banding pattern must be altered
- Can detect deletions, duplications, inversions, translocations, other abnormalities
- Deletions larger than 5 Mb are usually visible by standard karyotype (this is by microscopy)
  - Deletions that are not visible by karyotype are called “microdeletions”

# Classifying Chromosomes by Structure



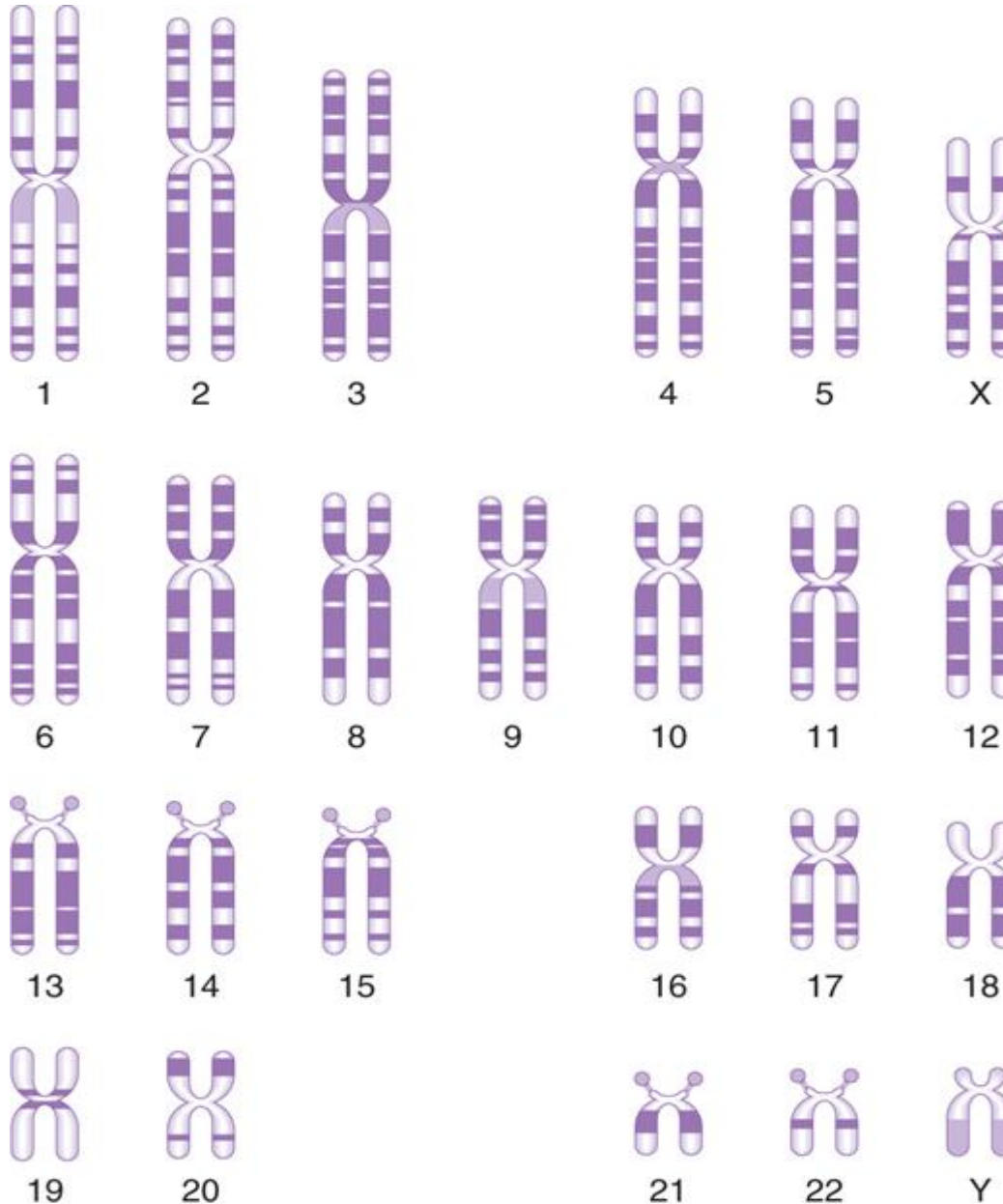
1) Total length of the chromosome

2) Position of the centromere

3) Presence of satellites

# Classifying Chromosomes by G-banding pattern

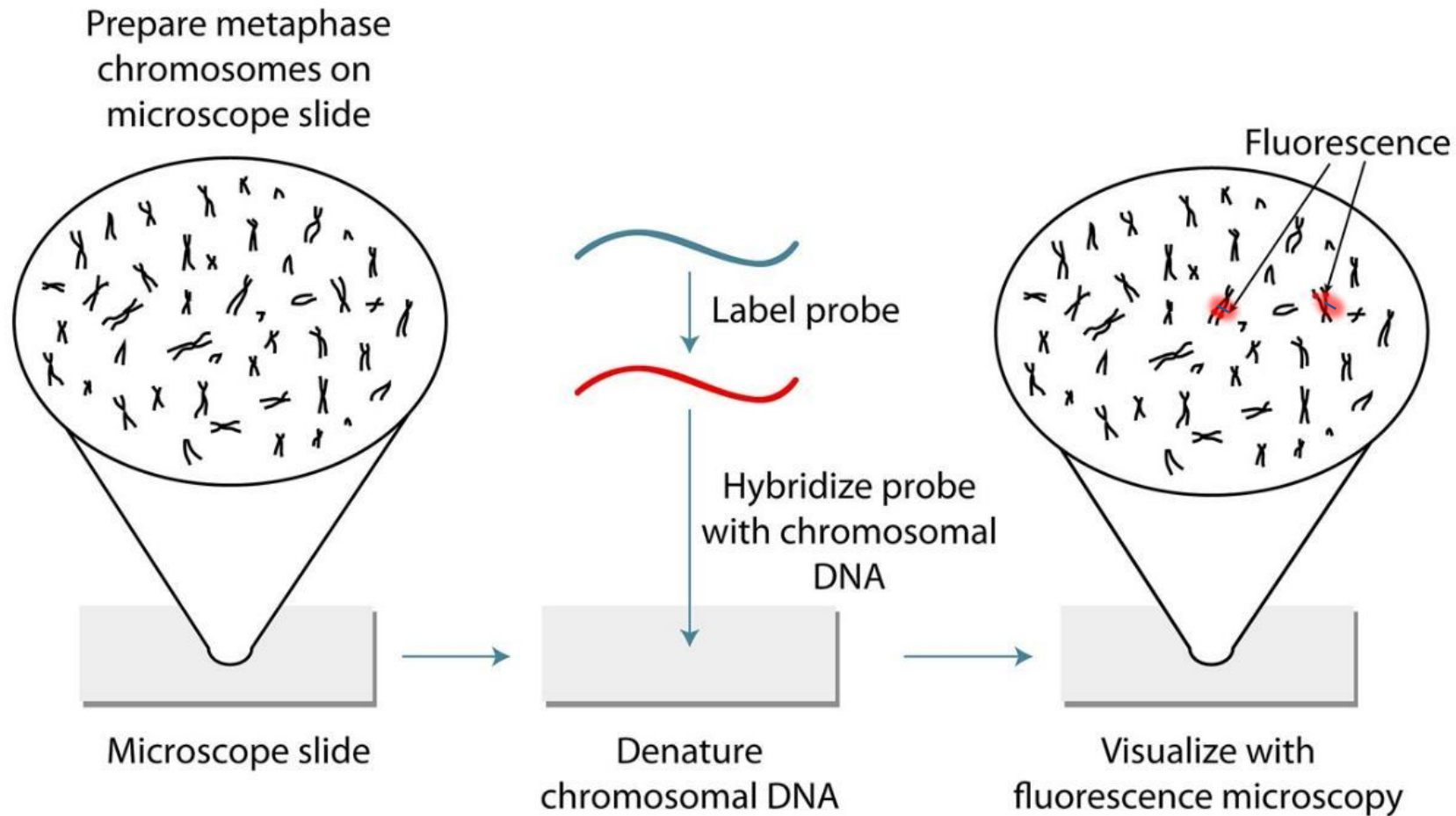
Sister chromatids,  
Only one chromosome  
each



Ideogram of the G-banding  
pattern of metaphase  
chromosomes stained with  
Giemsa

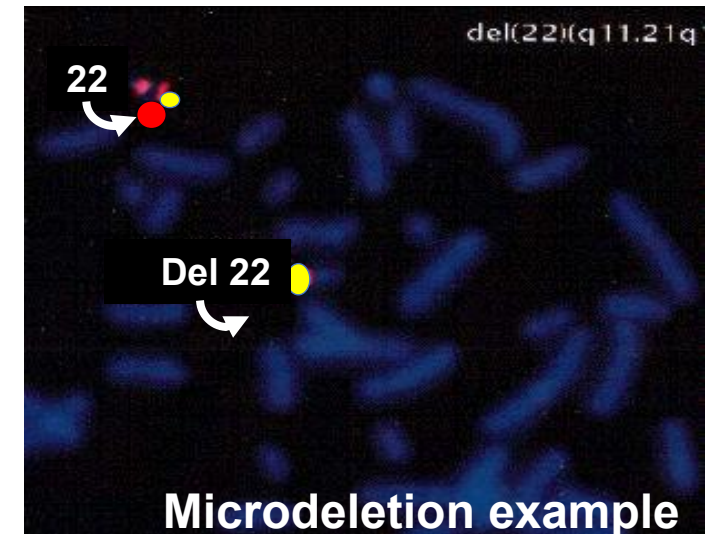
Resolution: can't detect  
deletions or insertions smaller  
than 5Mb

# Fluorescence *in situ* Hybridization (FISH)



# FISH

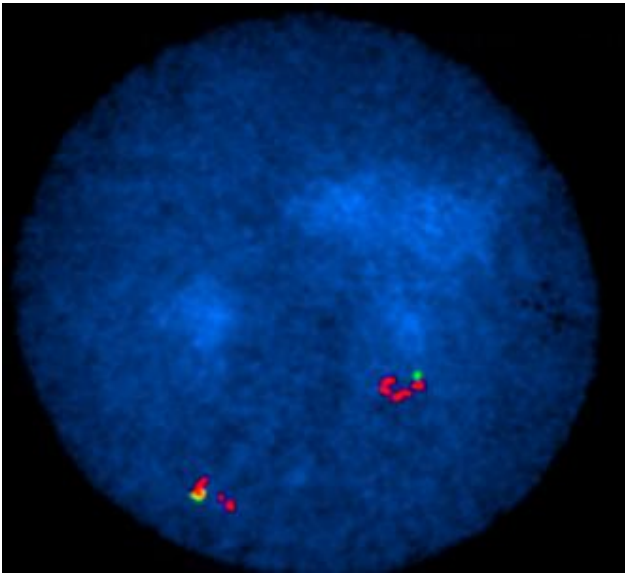
- **FISH probes are big; *ASO probes are small***
  - Typically to get a good signal, they are about **100 kb** or bigger
  - This allows a lot of fluorescent tags to be packed onto the probe
- With the correct probe FISH can detect:
  - Numerical chromosome aberrations
  - Deletions
  - Translocations
  - Gene amplification (very important in cancer characterization)
- FISH may detect many “microdeletions” because FISH is of higher resolution than G-band karyotype
  - As long as the deletion is larger than the probe being used, the deletion can be detected



# FISH

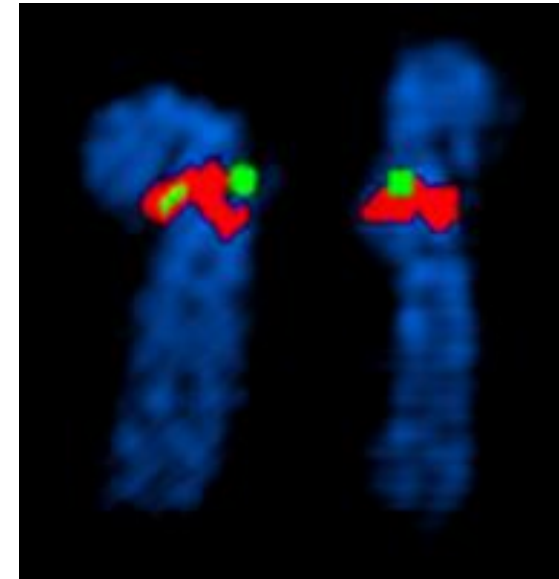
- Interphase stage FISH

- Rapid test
- Avoids need for cell culture
- Chromosomes are decondensed in the nucleus



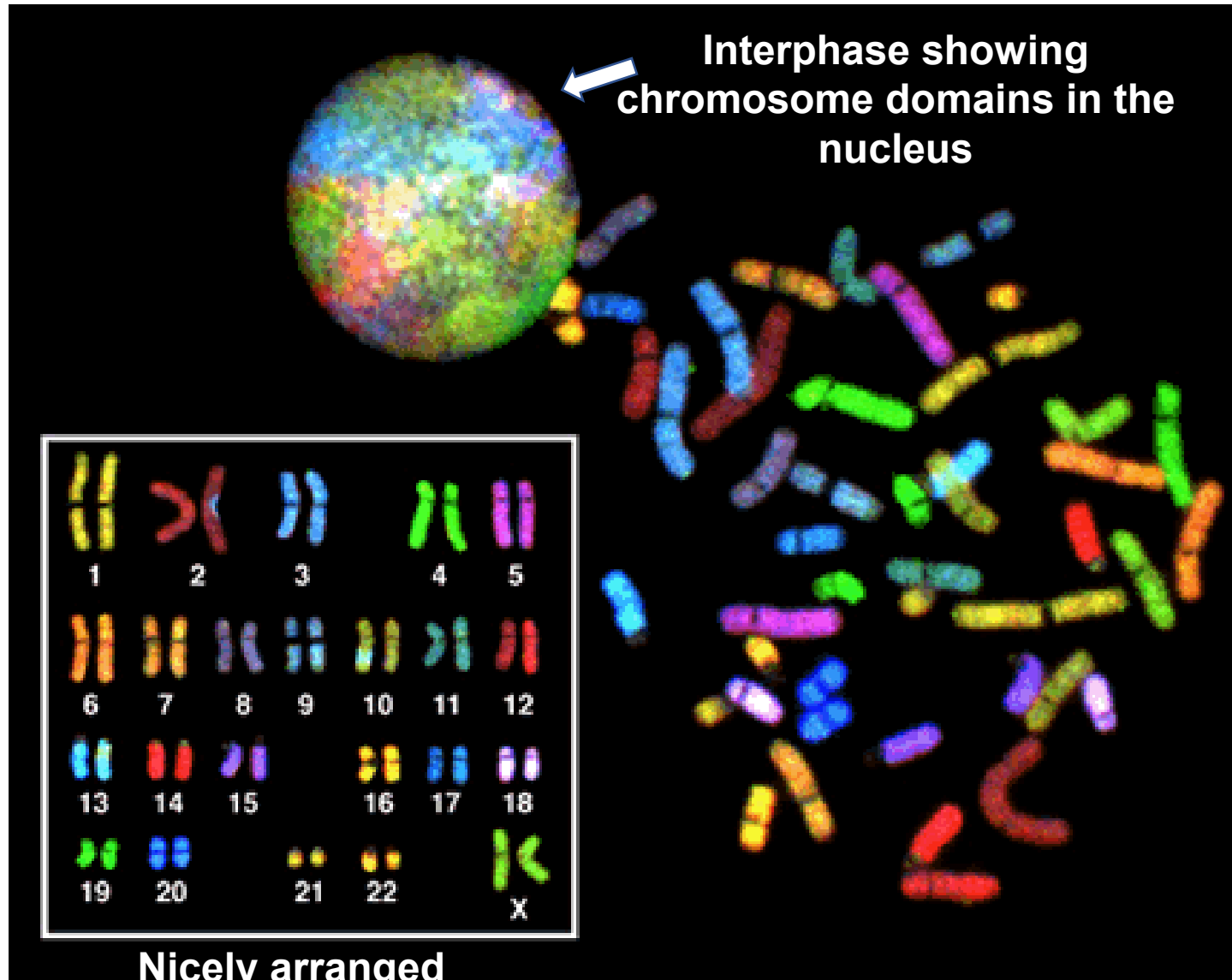
- Metaphase stage FISH

- Must first culture the cells
- Usually done after a G-band karyotype (condensed metaphase chromosomes).
- Gives location information because the banding pattern would be known





# Chromosome painting



- Chromosome painting, spectral karyotype, SKY FISH
  - Each chromosome probed so that it fluoresces a different color
- Can show translocations and rearrangements
- Very useful in **cancer genetics**
  - Genomic instability in advanced cancers



# Chromosomal abnormalities

- Chromosomal abnormalities account for a large number of spontaneous abortions
  - (noninduced embryonic or fetal death or passage of products of conception before 20 weeks gestation)
- Chromosomal abnormalities may be
  - Numerical
  - Structural

# Numerical Chromosome Abnormalities

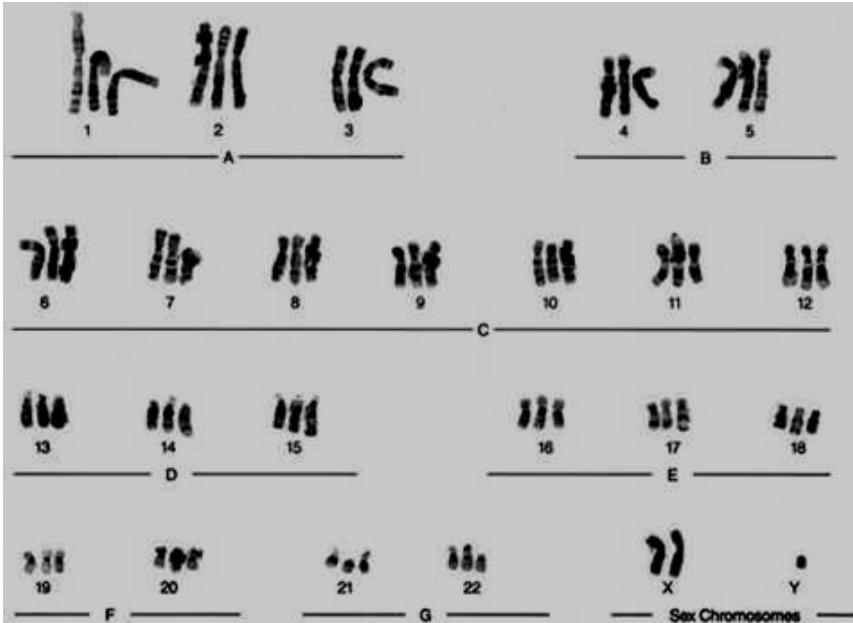
## **Euploidy (polyploidy)** (Number of chromosomes is in multiples of 23)

- Diploidy: Normal human complement of chromosomes, two of each (46)
- Triploidy: contain **three** copies of each chromosome (69)
  - Not compatible with life
- Tetraploidy: Contain **four** copies of each chromosome (92)
  - Lethal

## **Aneuploidy**

- Monosomy: Loss of a chromosome
  - Not compatible with life (Except monosomy X: Turner syndrome)
- Trisomy: Presence of an additional chromosome (only 3 are compatible with life)

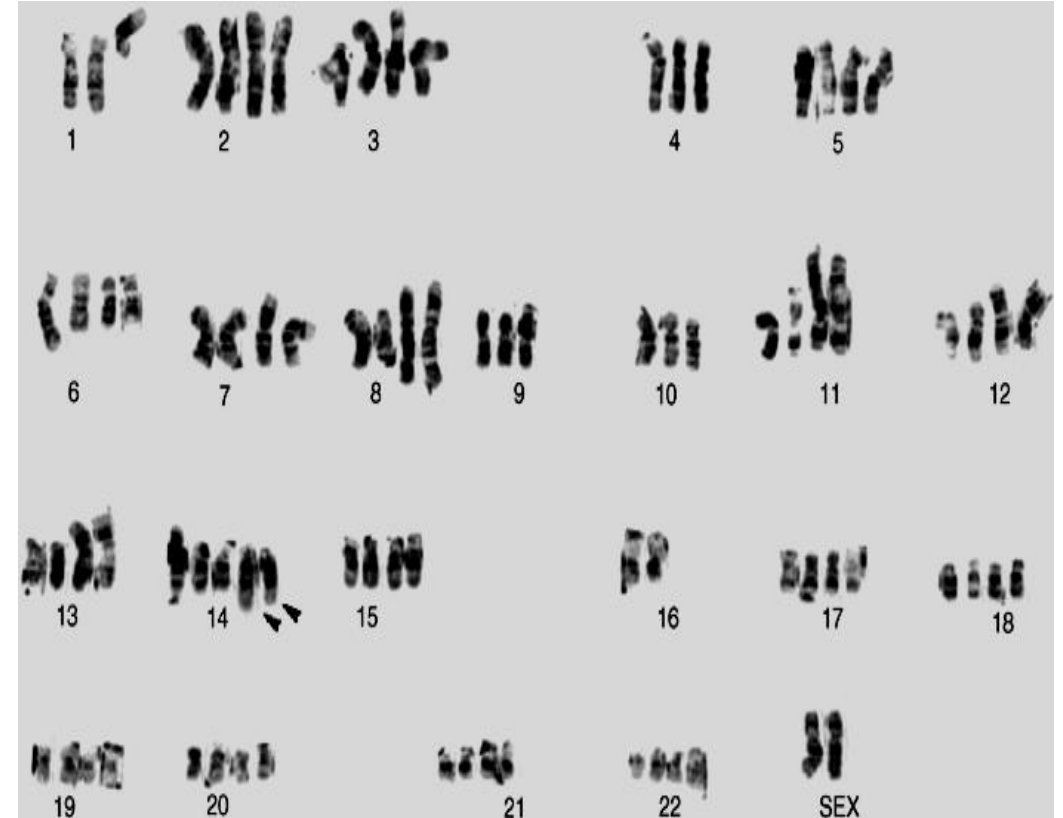
# Triploidy



69, XXY

Triploidy observed  
in an abortus

# Near Tetraploidy in Cancer



# Numerical Chromosome Abnormalities: Aneuploidy

- **Aneuploidy** (Chromosome number is not a multiple of 23)
  - Monosomy: Loss of a chromosome. Not compatible with life (Except monosomy X/Turner syndrome)
  - Trisomy: Presence of an additional chromosome [frequently confused with triploidy (69 chromosomes)]

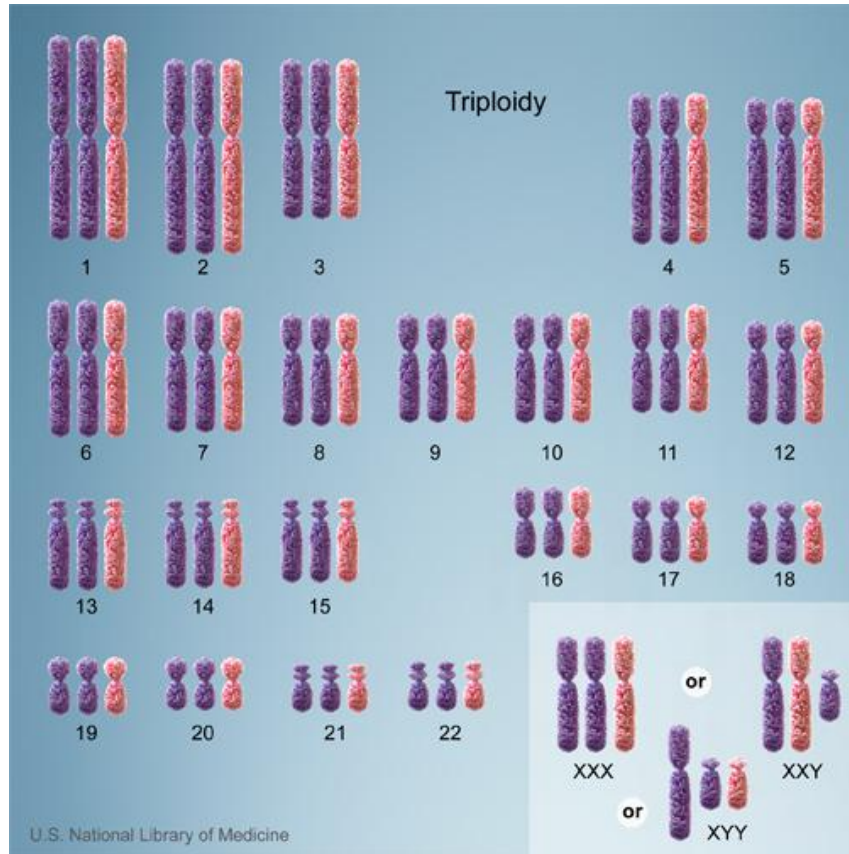
- Trisomy 21 (Down syndrome)
- Trisomy 18 (Edward syndrome)
- Trisomy 13 (Patau syndrome)

**Autosomal trisomy**  
(Chromosome number 47)

- Turner Syndrome (45,X)
- Klinefelter Syndrome (47,XXY)

**Sex Chromosome  
Aneuploidy**

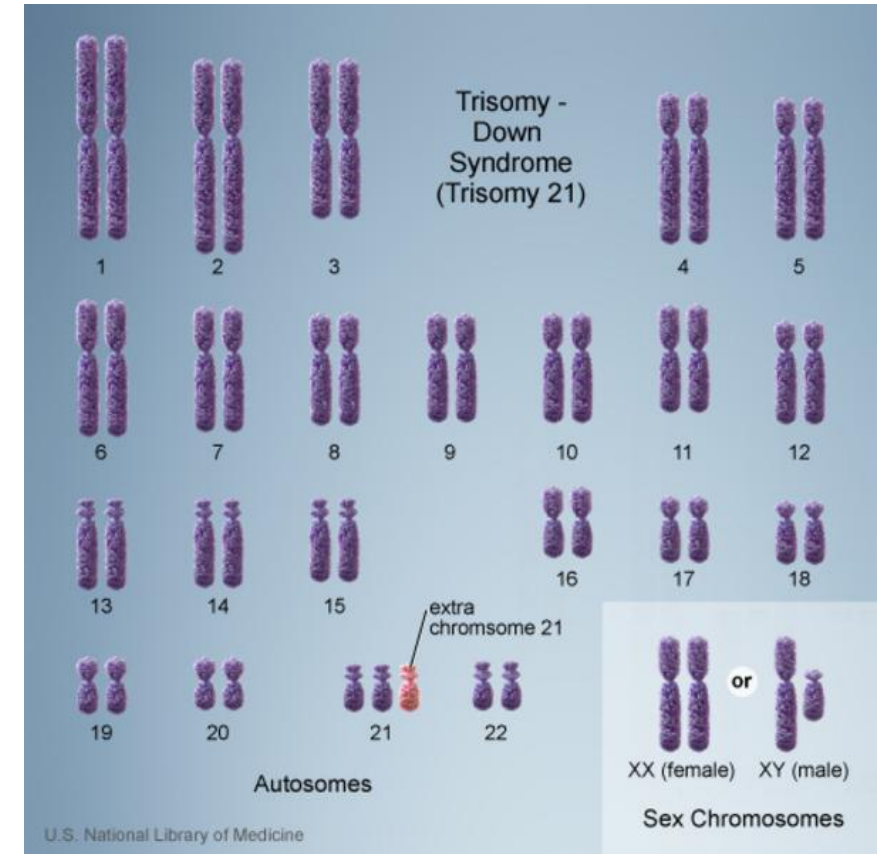
# Triploidy



**69,XXX or 69XXY or 69XYY**

Not compatible with life; Spontaneous Abortions

# Trisomy



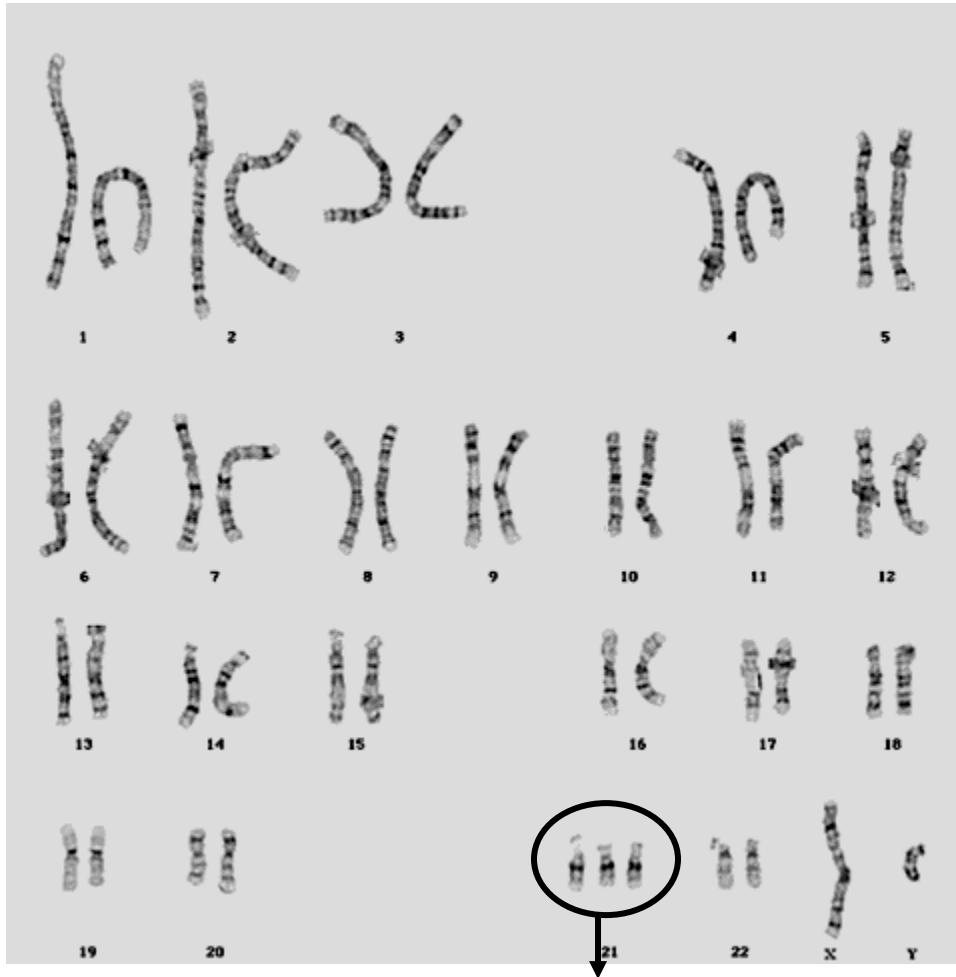
**47,XX or 47XY**

# Trisomy 21 (Down syndrome)

- Most common liveborn autosomal trisomy
- Risk factor: Increased maternal age, increases risk of meiotic nondisjunction during oogenesis (**most common is meiosis I nondisjunction**)
- Features:
  - Intellectual disability
  - Short stature
  - Depressed nasal bridge, upslanting palpebral fissures, epicanthal folds
  - Congenital heart defects
  - Single palmar crease
  - Develop changes similar to Alzheimer disease at a relatively young age.
    - One of the genetic factors responsible for Alzheimer is localized to chromosome 21 (amyloid precursor protein *APP*)

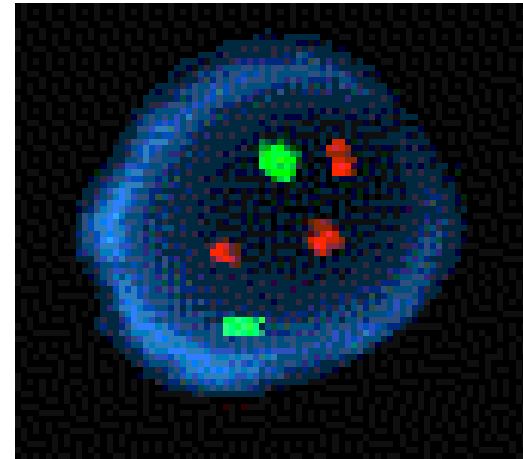


# Karyotype of Trisomy 21



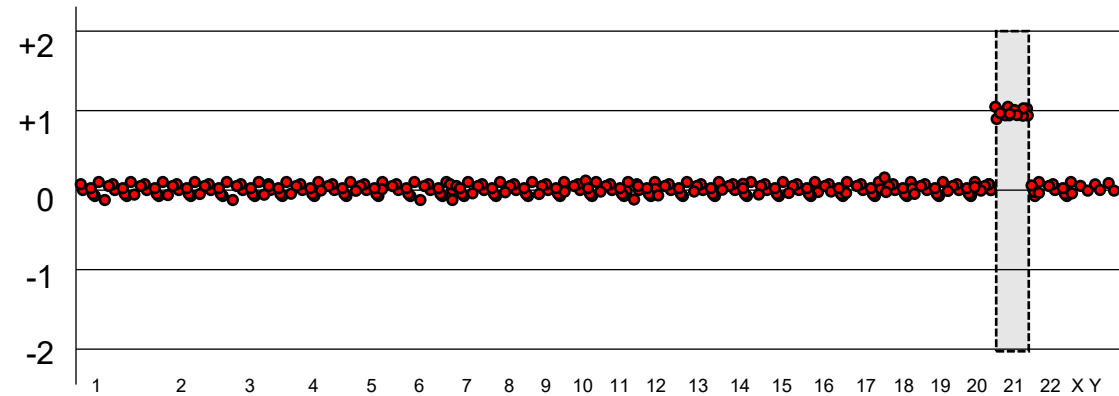
**47,XY,+21**

**Chromosome 21**



**FISH probes** will work with interphase stage cells to allow rapid return of results

- 2 chr 13 signals (green)
- 3 chr 21 signals (red)
- Prenatal diagnosis with maternal serum screening and ultrasound
- **Karyotype analysis or FISH with fetal cells is confirmatory**



Array CGH of all autosomes showing trisomy 21

# Down syndrome – genetic causes

- 1. Trisomy 21 - Due to non-disjunction during meiosis**
2. Some children may be mosaic – Some cells have trisomy 21; some other cells may have normal karyotype
- 3. Down syndrome may also result from Robertsonian translocation*

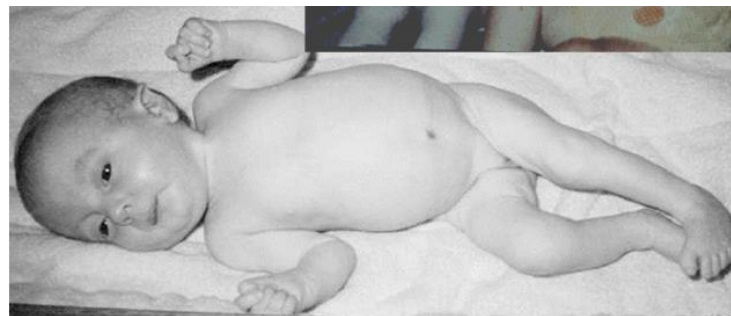


# Trisomy 18 (Edward syndrome)

Genetic mechanism is nondisjunction during **oogenesis**

## Features:

- Clenched fist, overlapping of fingers
- Rocker bottom feet
- Congenital heart defects
- Low-set ears, small lower jaw (micrognathia)
- Microcephaly
- Intellectual disability





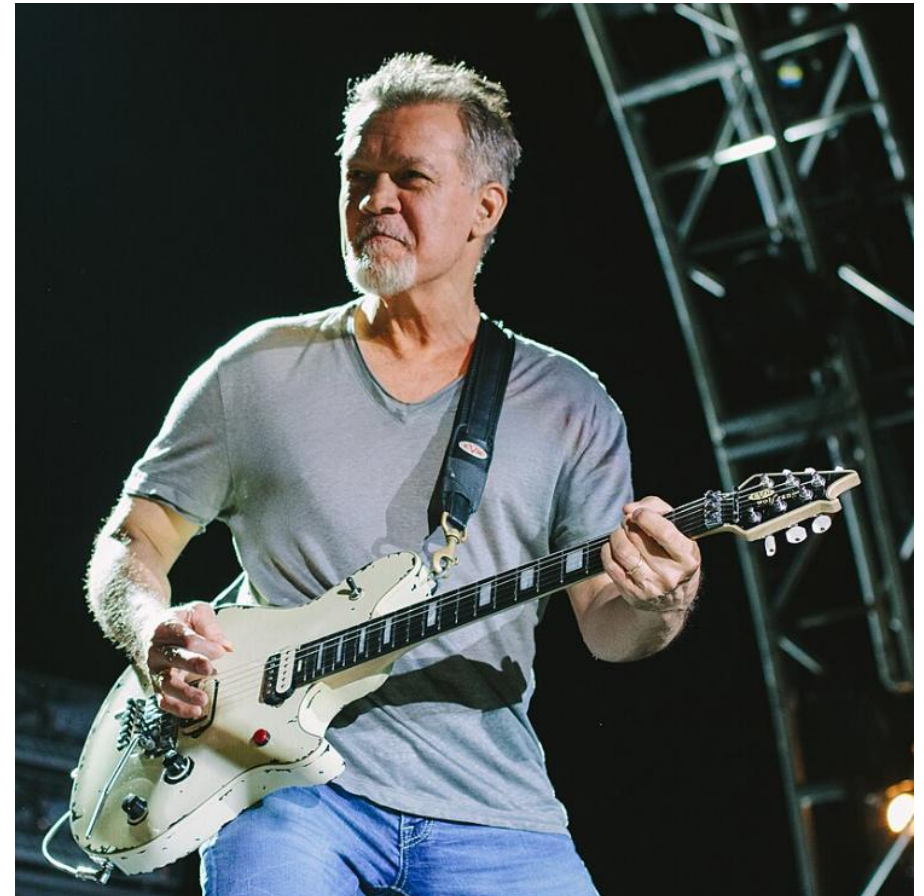
At 18 years old

Eddie Van Halen

Eddie's **overlapping fingers**

Rolling stone: 4<sup>th</sup> Greatest guitarist of all time

Rock and roll: **Rocker-bottom feet**



# Karyotype of Trisomy 18

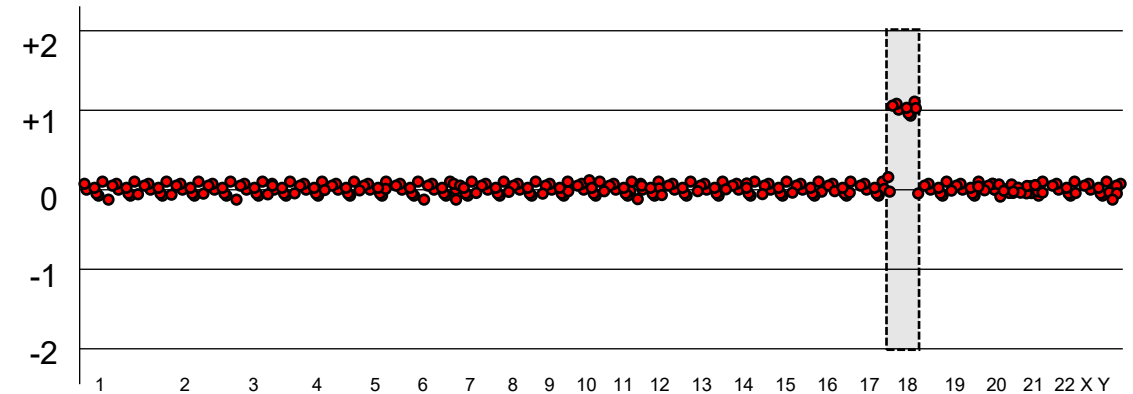


**47,XY,+18**

Chromosome 18

**FISH probes** will work with interphase stage cells to allow rapid return of results

- 3 chr 18 signals



Array CGH of all autosomes showing trisomy 18



# Trisomy 13 (Patau syndrome)

Most common genetic mechanism is nondisjunction during oogenesis

Features:

- Polydactyly
- Cleft lip and palate
- Microphthalmia
- Microcephaly
- Intellectual disability
- Cardiac anomalies (VSD or ASD)

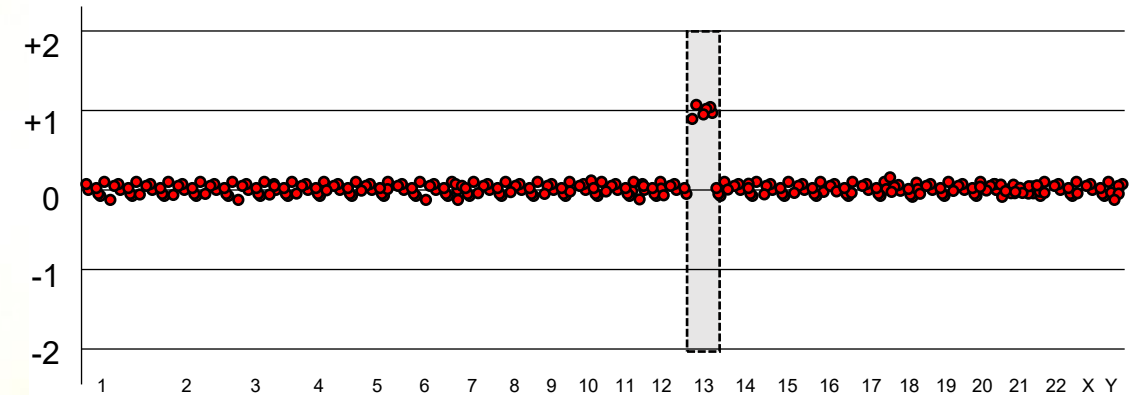
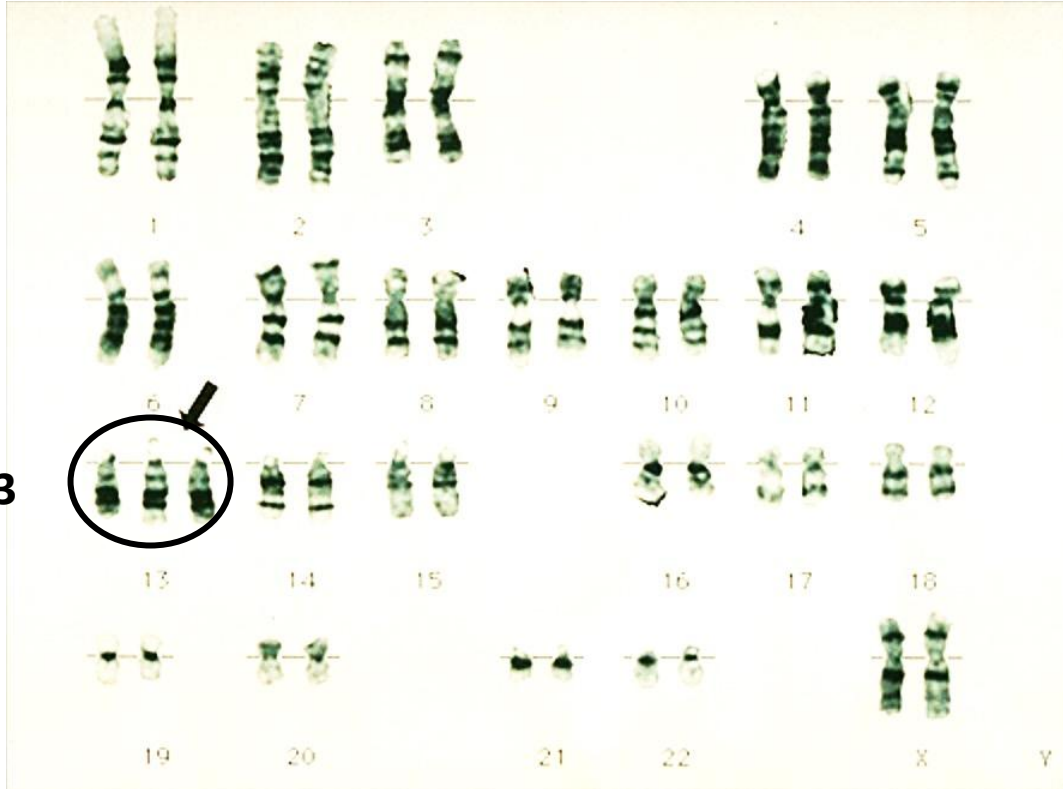


# Karyotype of Trisomy 13

**FISH probes** will work with interphase stage cells to allow rapid return of results

- 3 chr 13 signals

Chromosome 13



Array CGH of all autosomes showing trisomy 13

**47,XX,+13**

# Sex Chromosome Aneuploidy

- Turner Syndrome (45,X)
- Klinefelter Syndrome (47,XXY)



# Turner Syndrome (45,X)

- X chromosome monosomy
  - Usually due to nondisjunction during *spermatogenesis*
- Short stature
- Webbed neck, cystic hygroma at birth (neck swelling)
- Primary amenorrhea
- Gonadal dysgenesis
- 'Streak ovaries'
- Lymphoedema of hands and feet

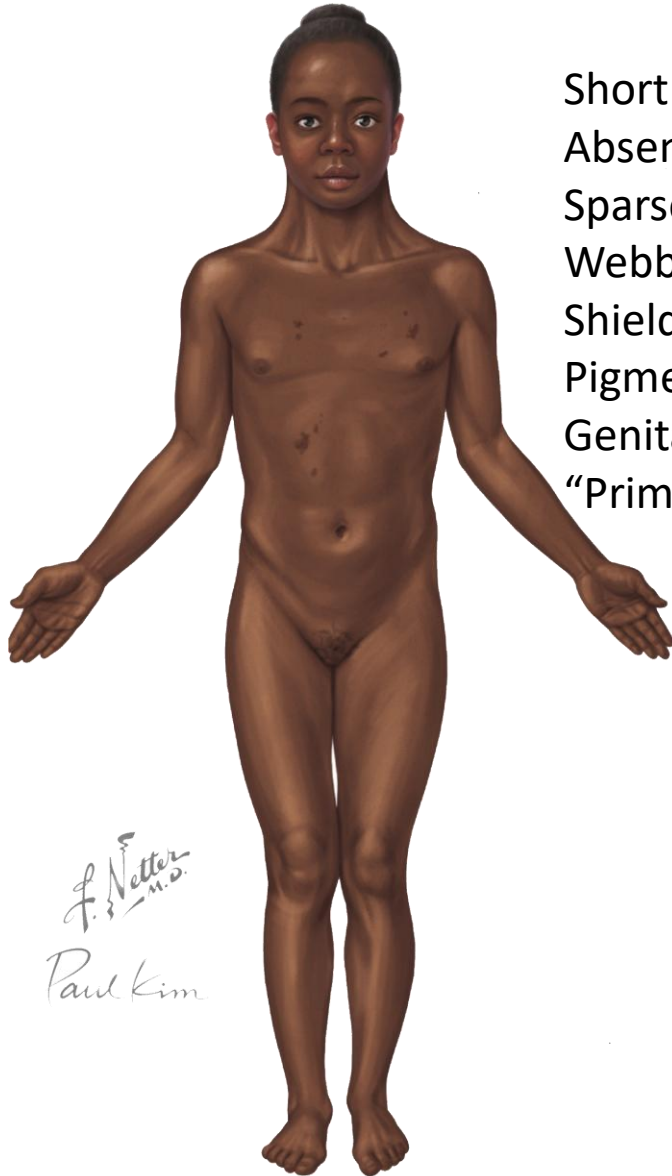


[http://www.dailyhome.com/view/full\\_story/5548416/article-A-unique-teenager](http://www.dailyhome.com/view/full_story/5548416/article-A-unique-teenager)

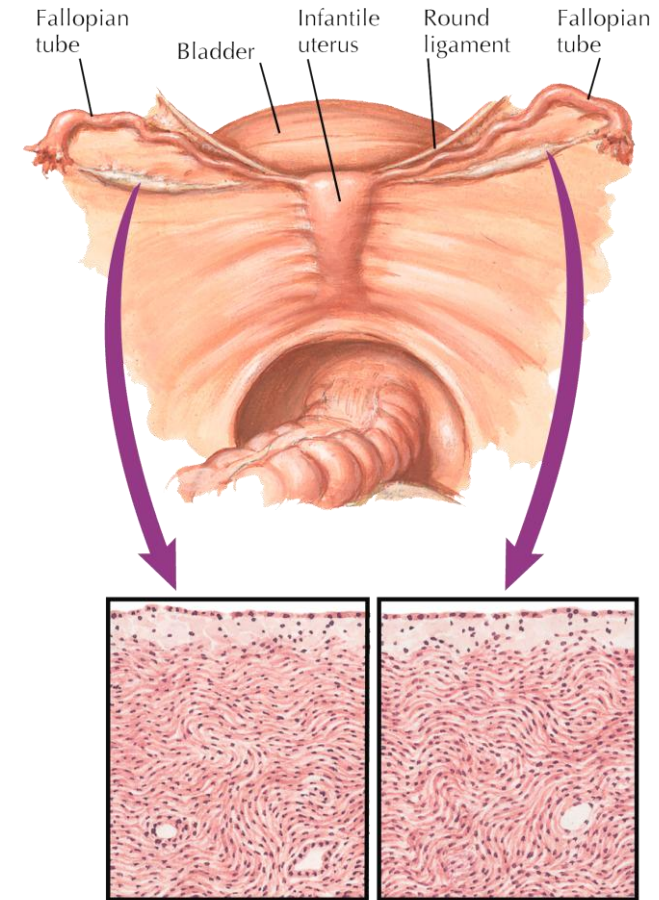


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# Turner Syndrome (45,X)



Short stature  
Absence of secondary sex characteristics  
Sparse pubic hair  
Webbed neck  
Shield-like chest  
Pigmented nevi  
Genital streaks in place of gonads  
"Primary amenorrhea"



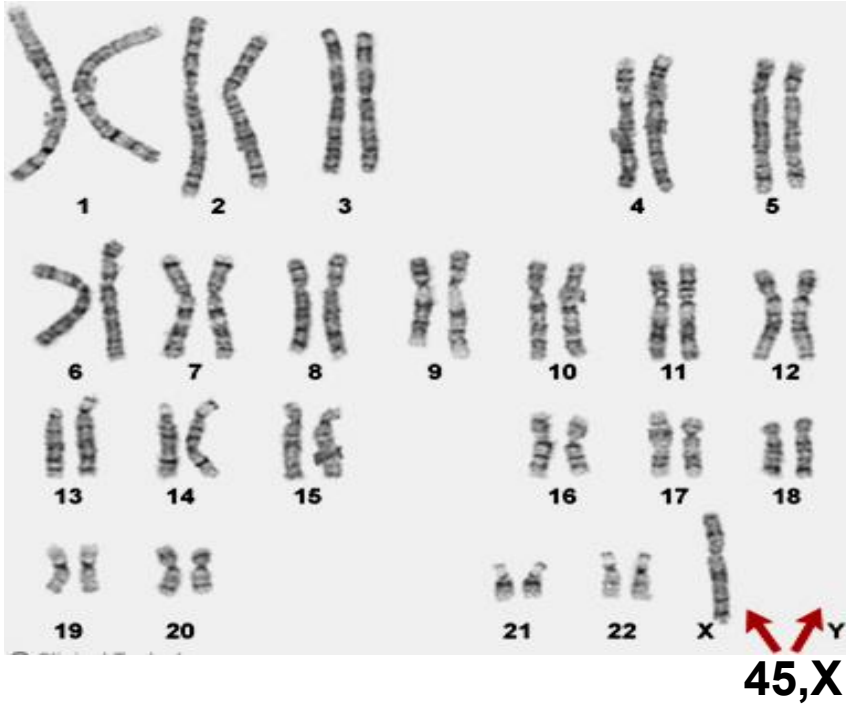
Primitive genital streaks in place of gonads,  
wavy stroma with absence of germinal elements

## Therapies:

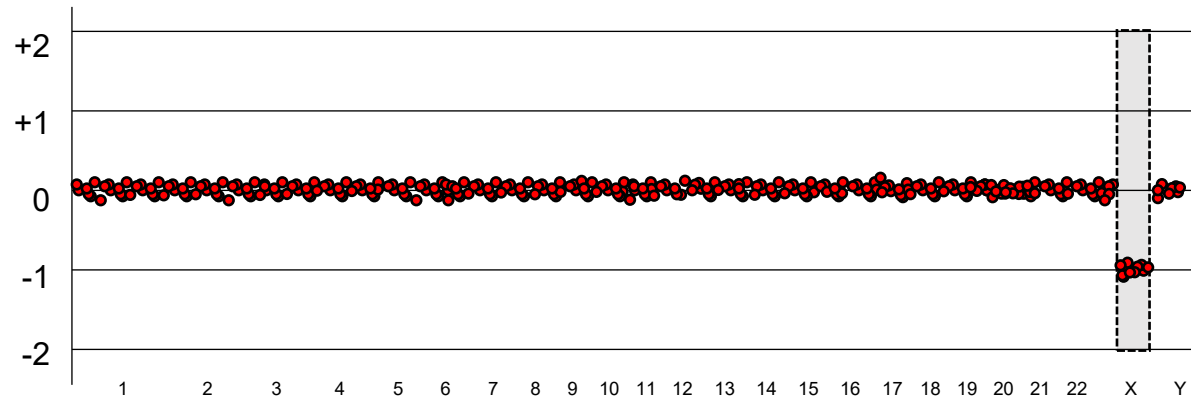
- Hormone replacement therapy
- Human growth hormone if diagnosis established before 10-years-old
- Assisted reproduction with donor egg is advancing



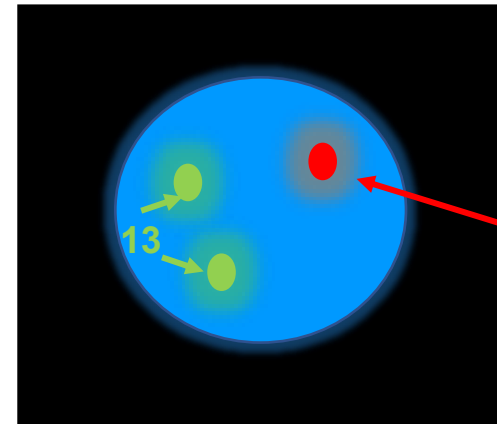
# Turner syndrome karyotype (45,X)



- Many TS patients are mosaic
  - Some of their cells are 45,X
  - Other cells are 46,XX and 47,XXX (indicate mitotic nondisjunction during embryogenesis)
- Cells with 45,X: Have **no Barr body** (Inactivated X-chromosome) (**Female with NO Barr body**)
- Isochromosome X will also lead to Turner syndrome (*discussed next lecture*)



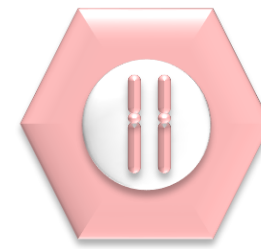
Array CGH of all chromosomes showing monosomy X  
(control DNA is female)



**FISH probes** will work with interphase stage cells to allow rapid return of results

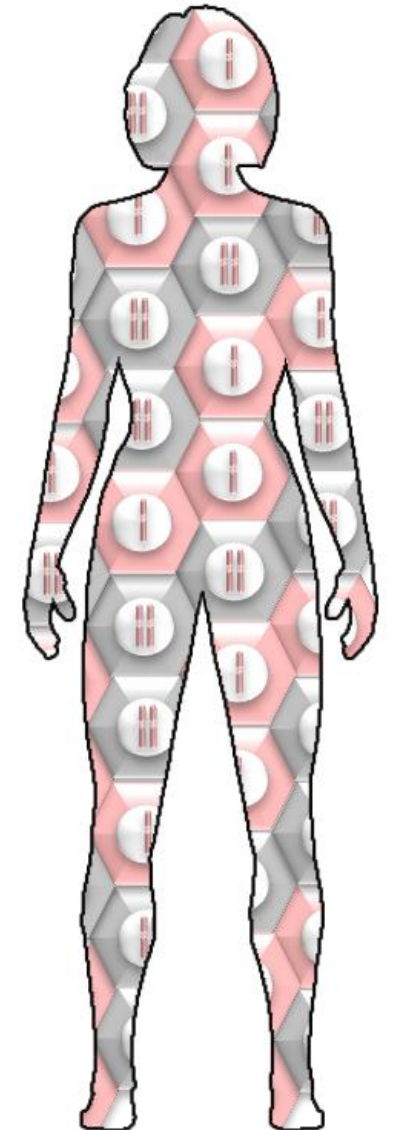
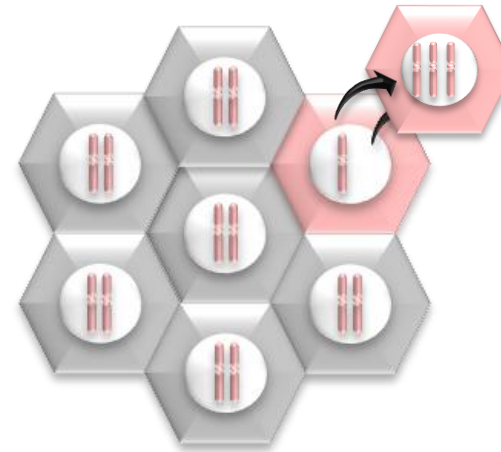
- 1 chr X signal

# Mosaic Turner Syndrome- 46, XX



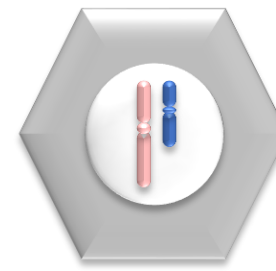
- **Some females are mosaic for Turner Syndrome**

- Fertilized egg, 46,XX
- Mitotic Nondisjunction during embryogenesis
- Some cells are 45,X others are 46,XX and 47,XXX

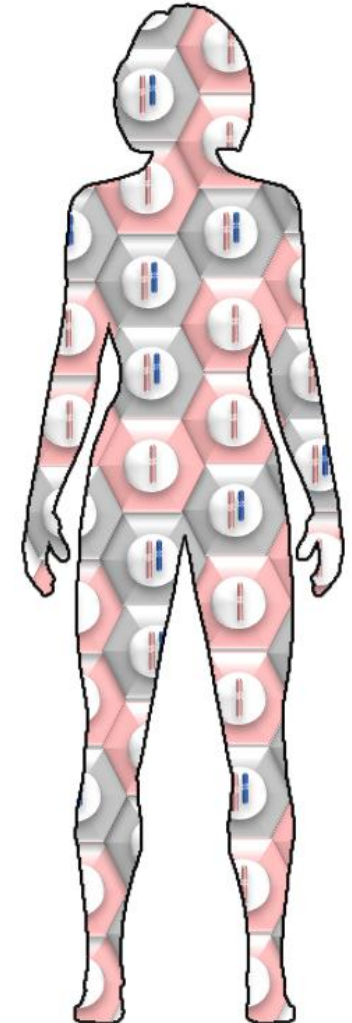
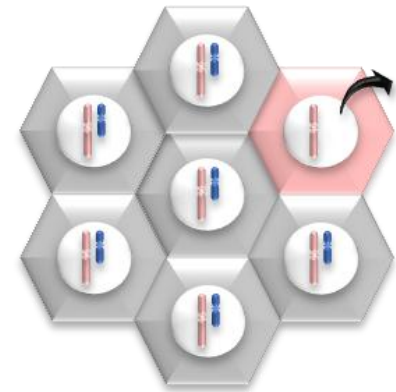


During a post-fertilization mitosis, one of the X chromosomes evicted during cell division

# Mosaic Turner Syndrome- 46,XY



- Some **phenotypic females** are mosaic
  - Fertilized egg 46,XY
  - Mitotic Nondisjunction during embryogenesis
  - Some cells are 45,X others are 46,XY
  - **Mosaic 45,X/46XY** are recommended for
  - **oophorectomy** due to risk of gonadoblastoma



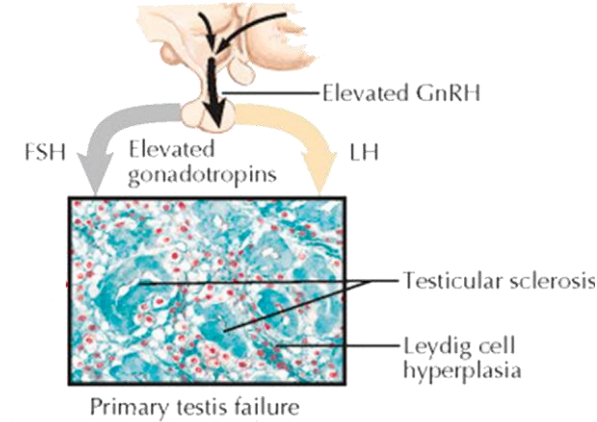
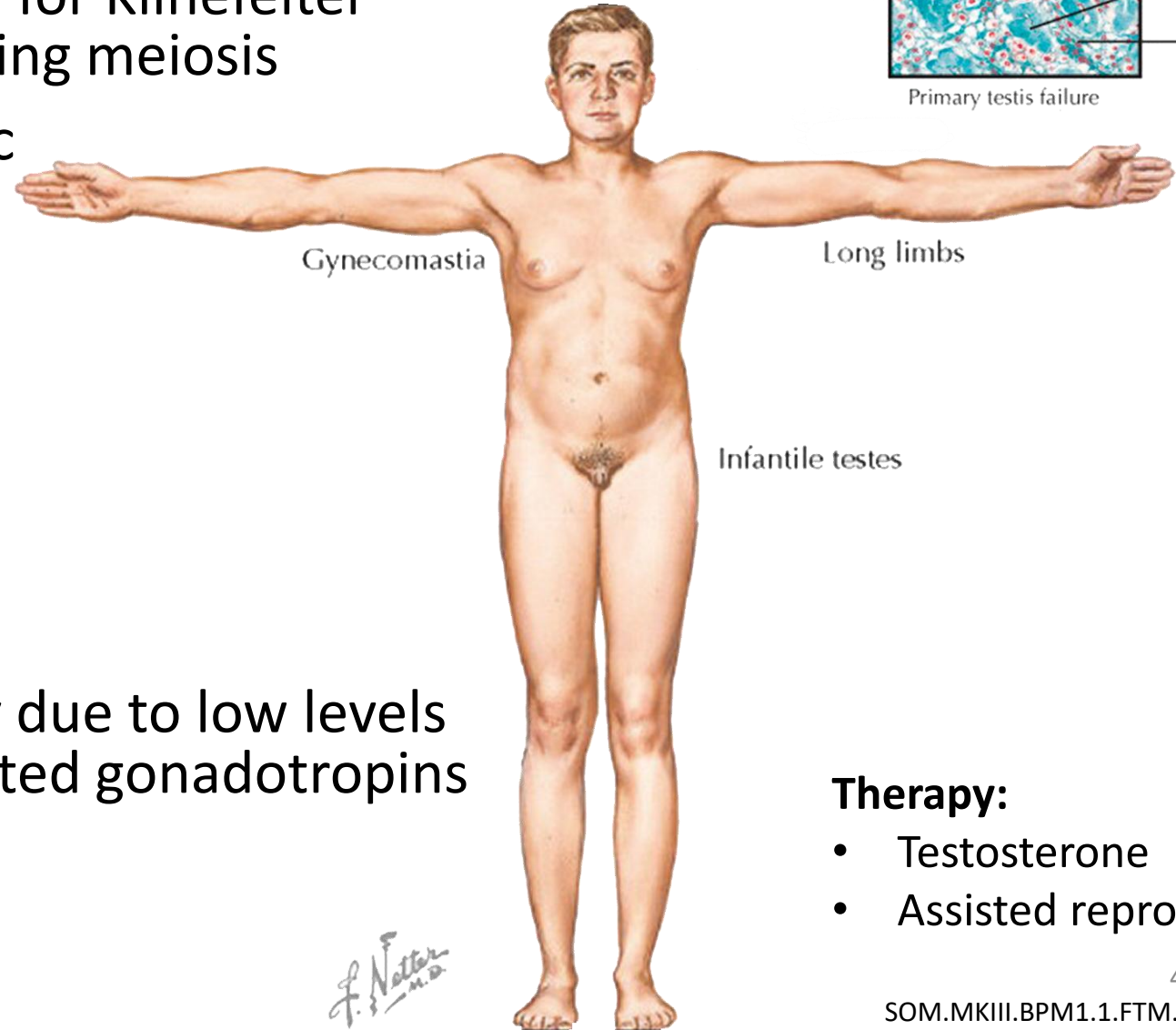
During a post-fertilization mitosis, the Y chromosome is evicted during cell division

# Klinefelter syndrome (47,XXY)

- Genetic mechanism responsible for Klinefelter syndrome is nondisjunction during meiosis
- Some individuals may be mosaic
  - (46XY/47XXY/ 48XXXY)

## Features:

- Tall with long limbs
- Gynecomastia
- Female distribution of hair
- Testicular atrophy and infertility due to low levels of testosterone leading to elevated gonadotropins
- Feminization of fat distribution
- Osteoporosis



## Therapy:

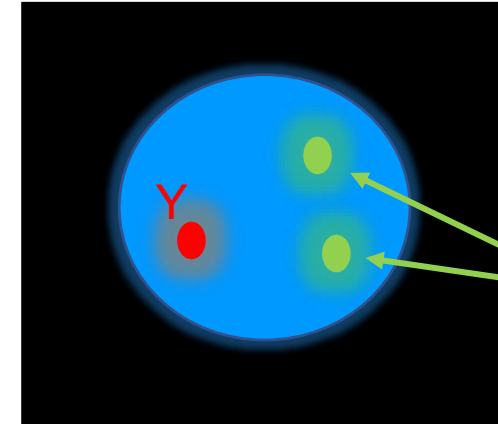
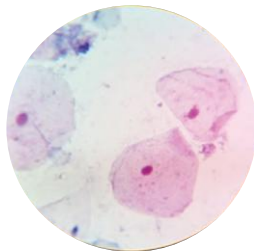
- Testosterone
- Assisted reproduction

# Klinefelter Syndrome (47,XXY)



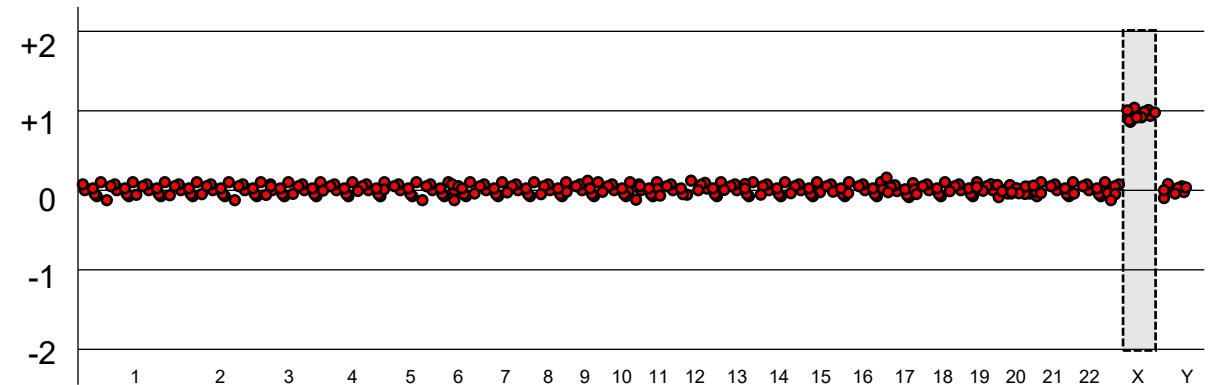
**47,XXY**

**Presence of a Barr body  
(X- chromosome inactivation) will  
be observed in the buccal mucosal  
cells**



**FISH probes** will work with  
interphase stage cells to  
allow rapid return of results

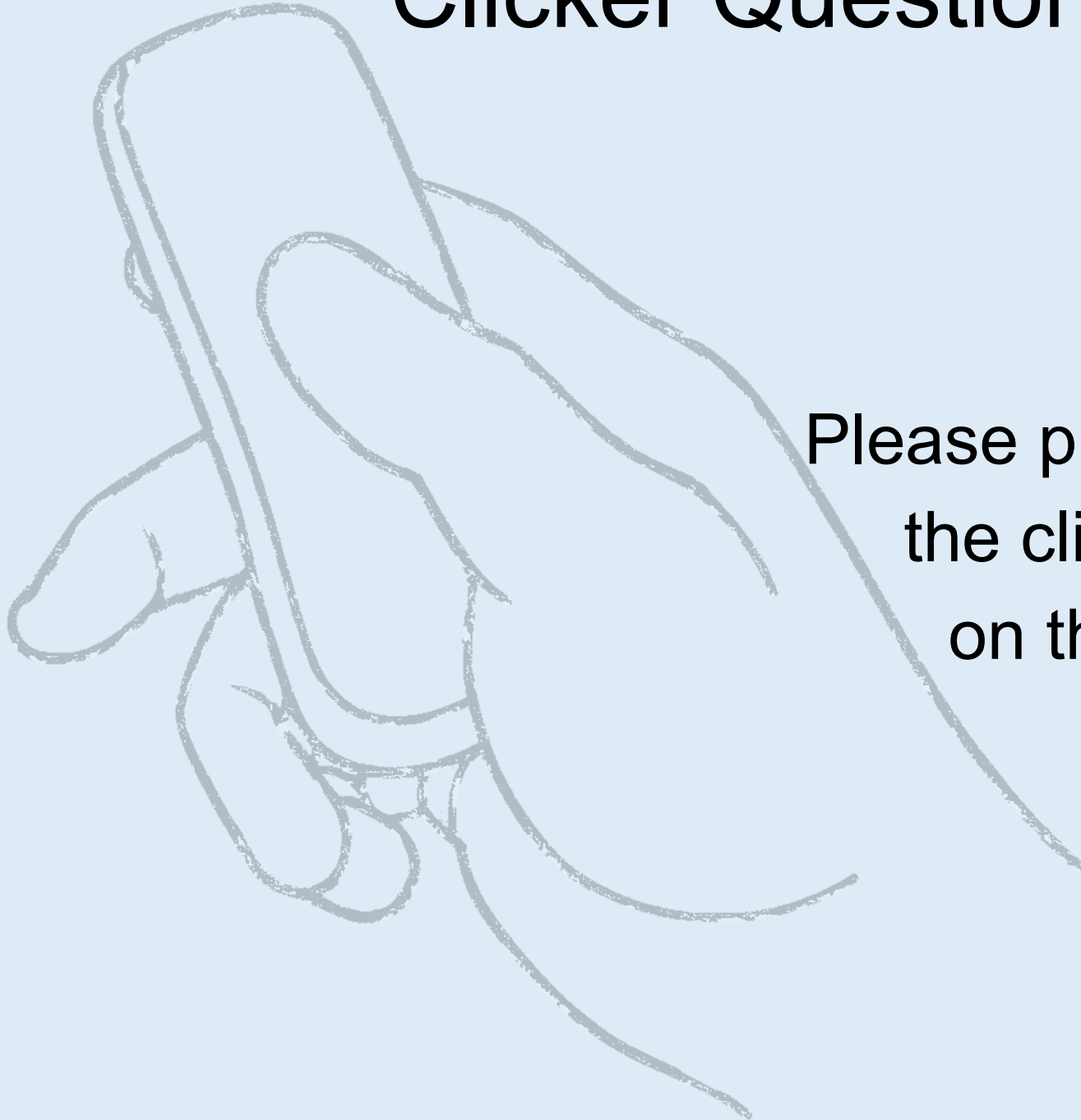
- 2 chr X signals



Array CGH of all chromosomes showing extra X  
(control DNA is male)



# Clicker Question Next!



Please prepare to answer  
the clicker question  
on the next slide



# Chromosomal Aneuploidy is thought to occur by Nondisjunction

|                        |                      |
|------------------------|----------------------|
| • Trisomy 21           | 1/830 live births    |
| • 47,XXY (Klinefelter) | 1/1000 live births   |
| • 45,X (Turner)        | 1/4000 live births   |
| • Trisomy 18           | 1/7,500 live births  |
| • Trisomy 13           | 1/22,700 live births |

*You do not need to memorize these numbers!*

Data are approximated from Hsu LYF: Prenatal diagnosis of chromosomal abnormalities through amniocentesis, 1998

# Meiosis and non-disjunction

Homologous chromosomes and DNA is duplicated

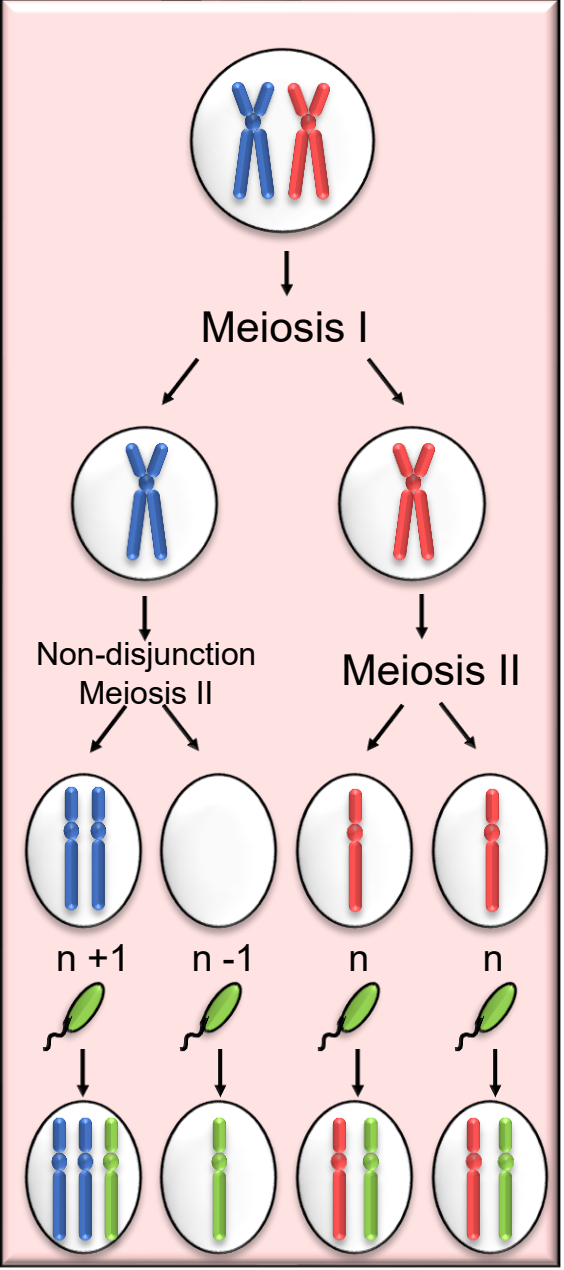
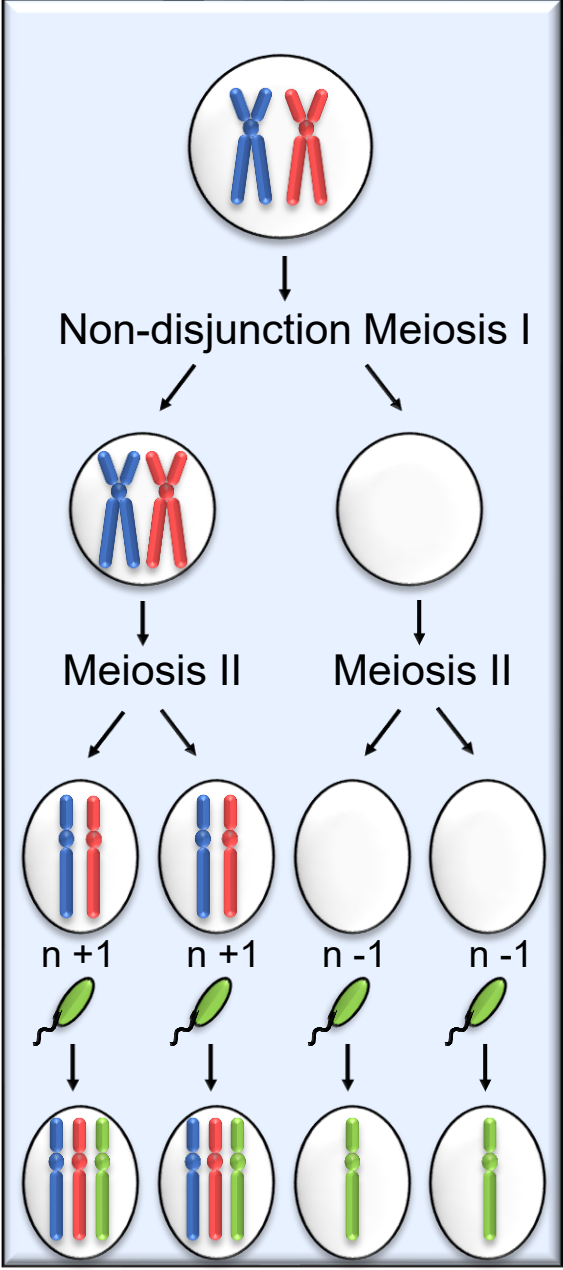
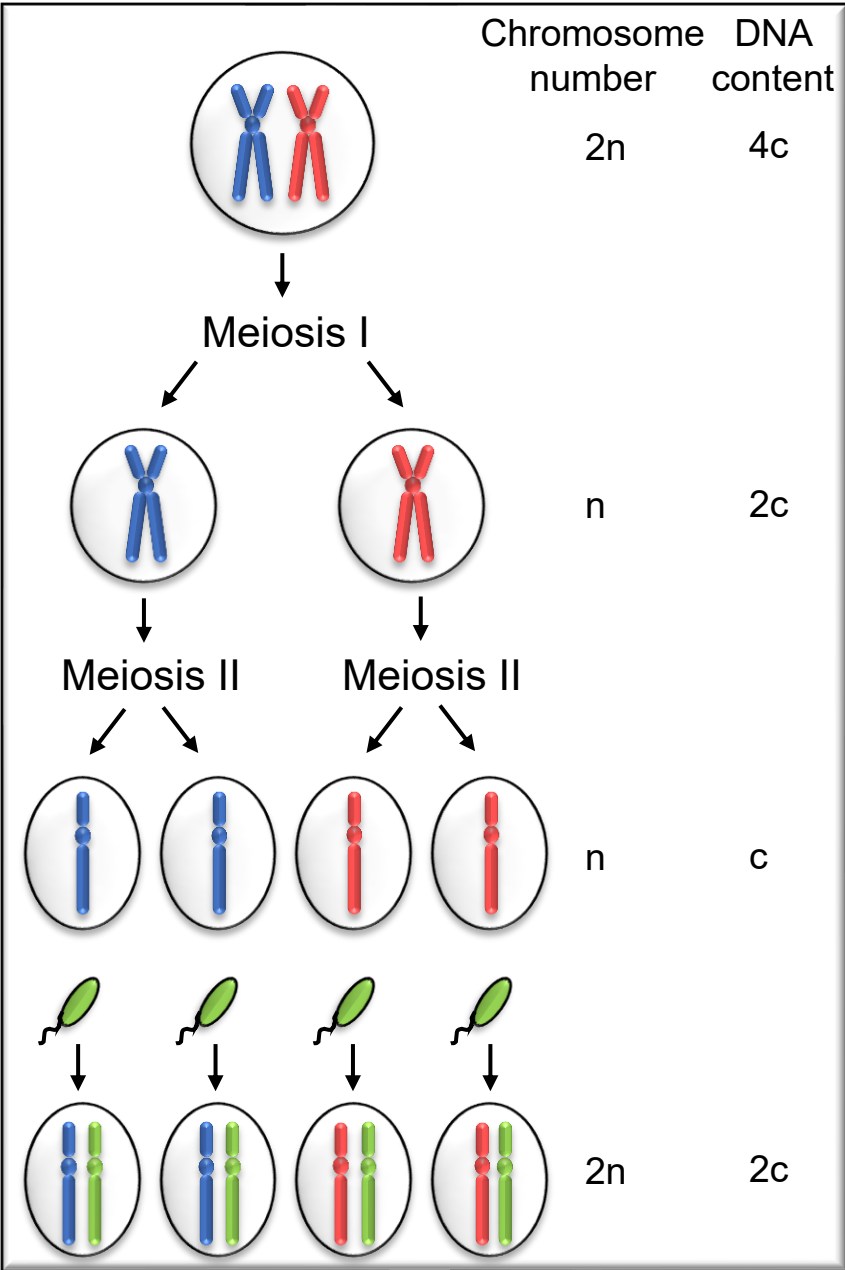
First Division

Second Division

Gametes

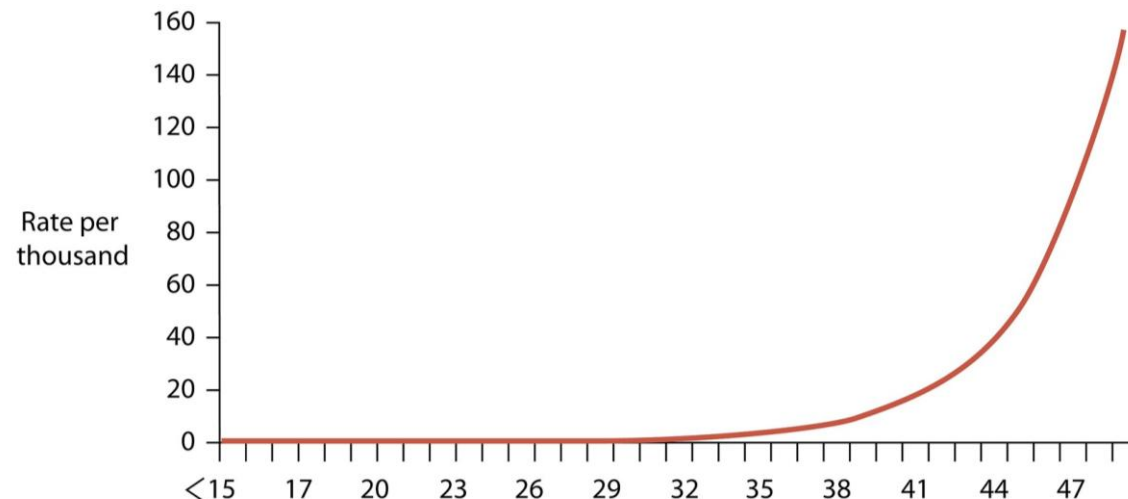
Fertilization

Zygote



# Non-disjunction

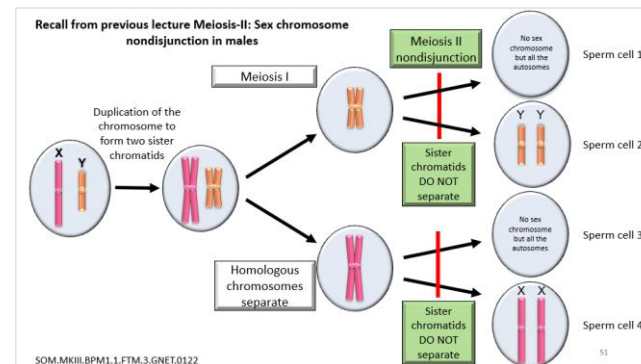
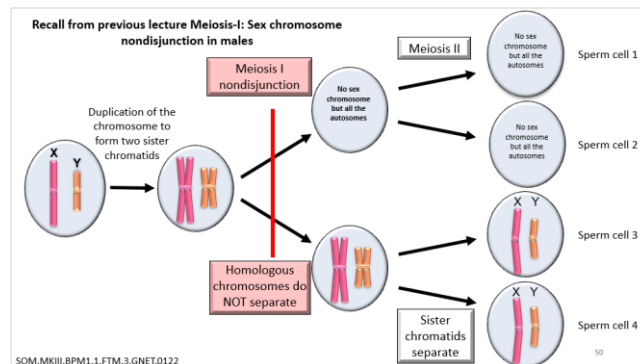
- Non-disjunction is most common in female meiosis I
- Individuals with Down syndrome usually have two maternal chromosomes 21 and one paternal chromosome 21.
- If two maternal chromosomes are not identical to each other, the non-disjunction occurred at meiosis I (that is they have different alleles shown as blue and red on previous slide)
- If the two maternal chromosomes are identical, the non-disjunction occurred at meiosis II (sister chromatid shown as two blue alleles on previous slide)



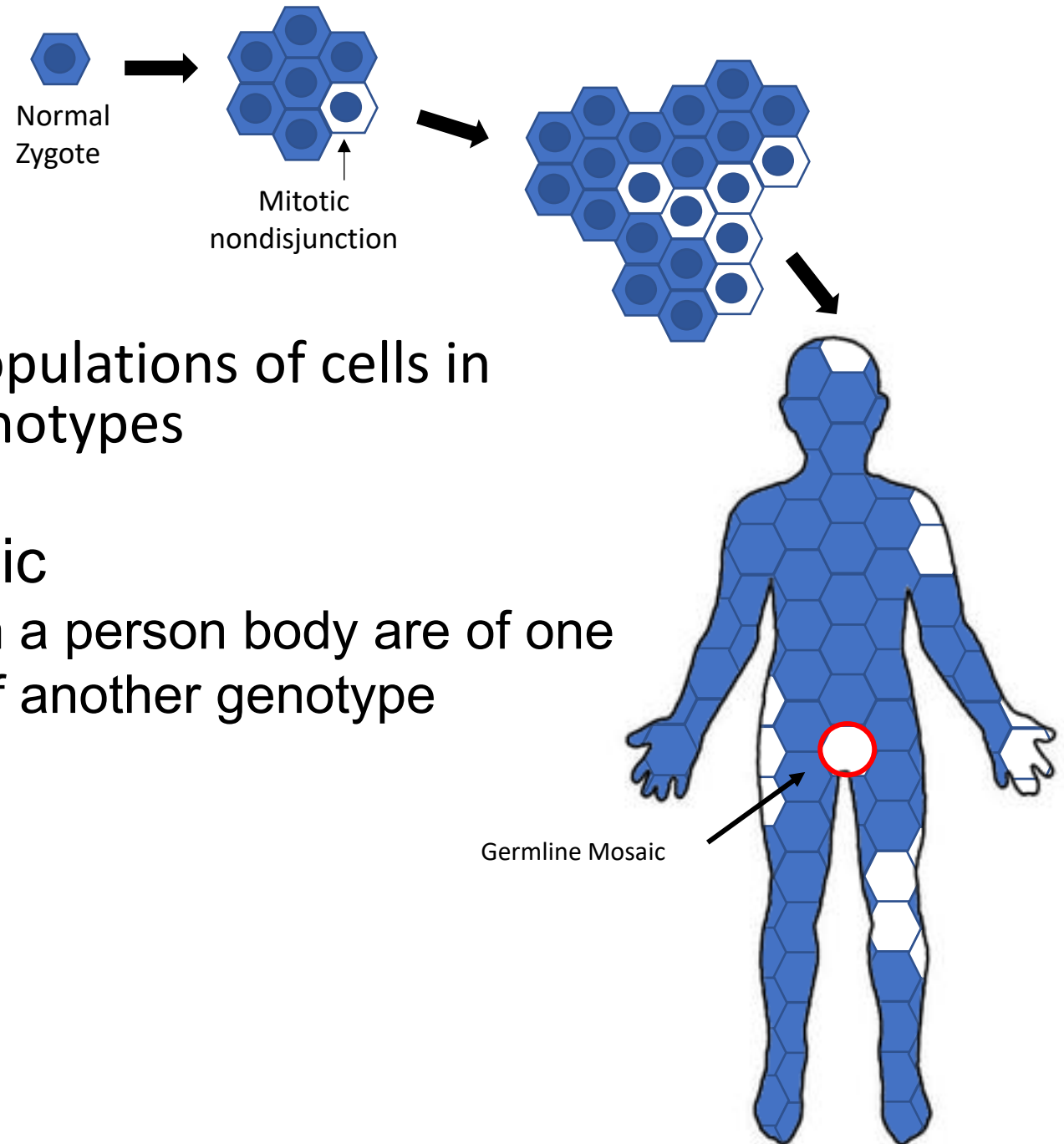
Increase in the risk of nondisjunction resulting in trisomy 21 with advanced maternal age

# Draw out which type of non-disjunction, *meiosis I or meiosis II*?

- 47, XXY Klinefelter syndrome may be caused by nondisjunction during meiosis I or II in the mom, or nondisjunction during meiosis I in dad
  - Explain why meiosis II nondisjunction in the father (sperm) cannot result in Klinefelter syndrome (hint: a person with Klinefelter syndrome is male)
- 47, XYY fetus is the result of meiosis II nondisjunction in dad
  - May not be caused by meiotic non-disjunction in the mom (hint: this person is also male)
- 47, XXX females may be the result of meiosis I or II nondisjunction in moms or meiosis II non-disjunction in dad
  - May not be caused by meiosis I non-disjunction in the dad



# MOSAICISM

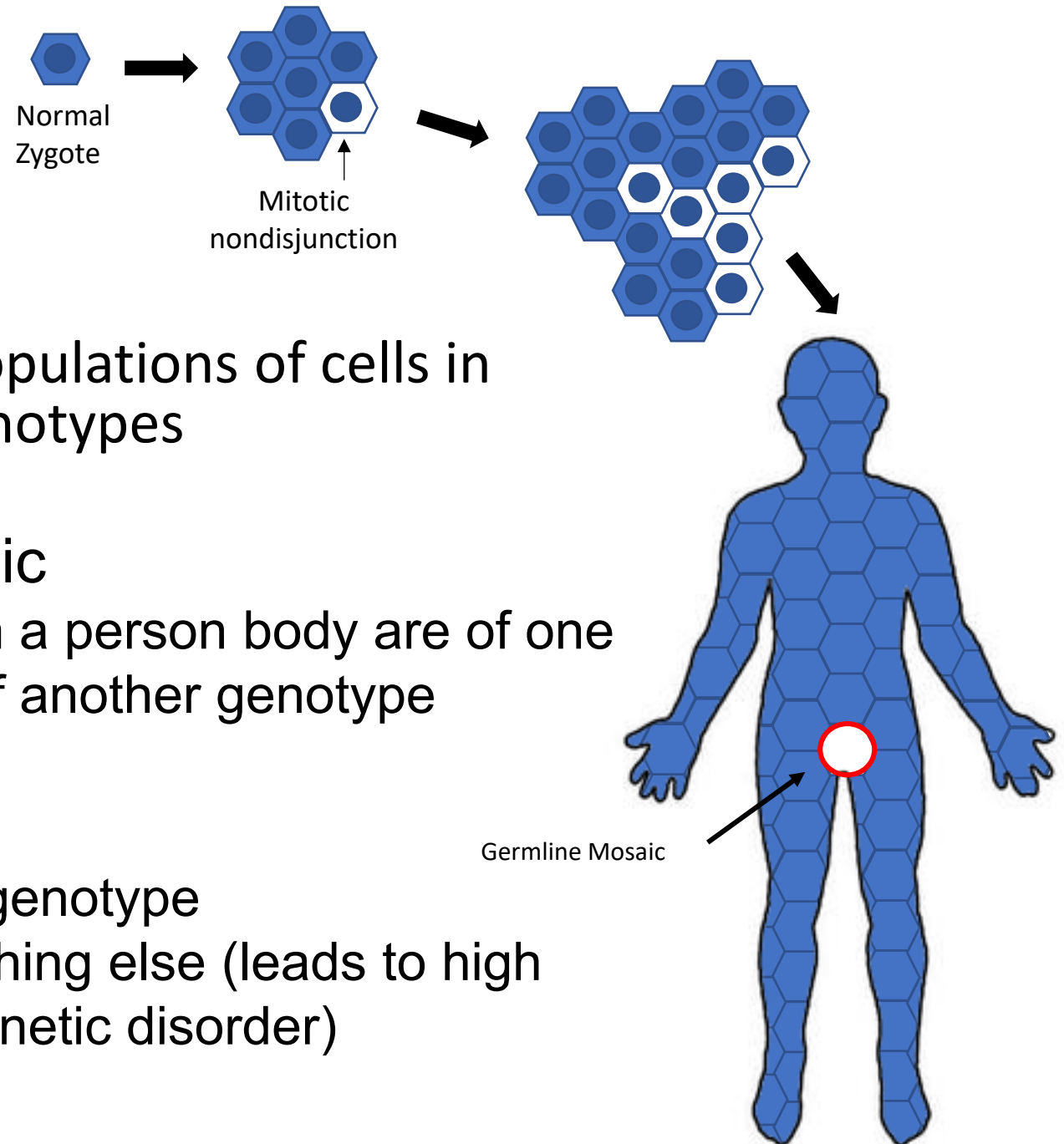


- The presence of two or more populations of cells in one individual with different genotypes

## Mixed somatic/germline mosaic

Some percentage of the cells in a person body are of one genotype, and other cells are of another genotype

# MOSAICISM



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## Mixed somatic/germline mosaic

Some percentage of the cells in a person body are of one genotype, and other cells are of another genotype

## Confined germline mosaic

All cells in the body are of one genotype

Cells of the gametes are something else (leads to high risk of having children with a genetic disorder)

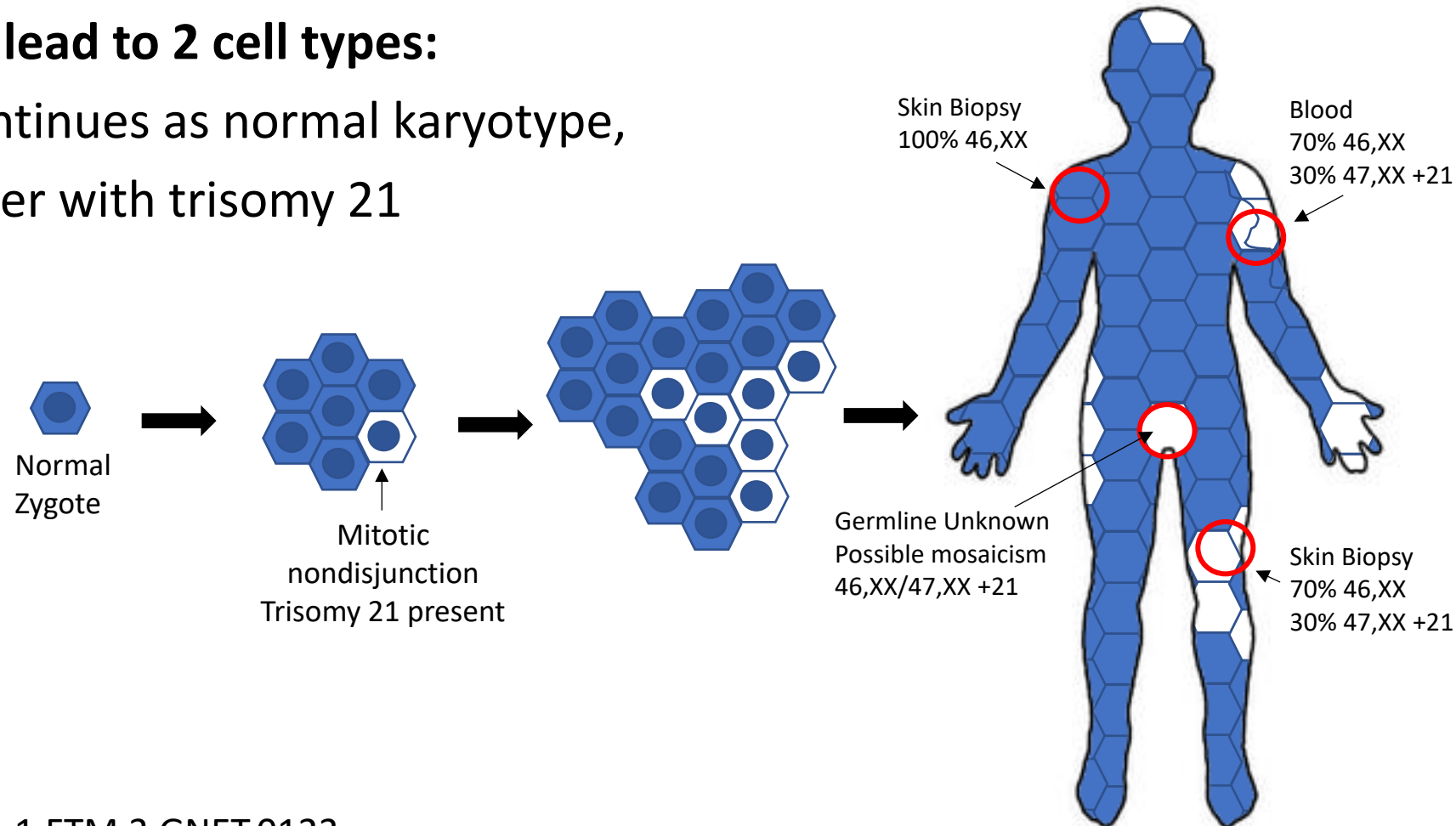


# Consider 2 ways that a person could be mosaic for a Down syndrome:

1. Non-disjunction may occur as a **post-zygotic** event during embryonic development; that is mitotically (after fertilization of a normal ovum and normal sperm)

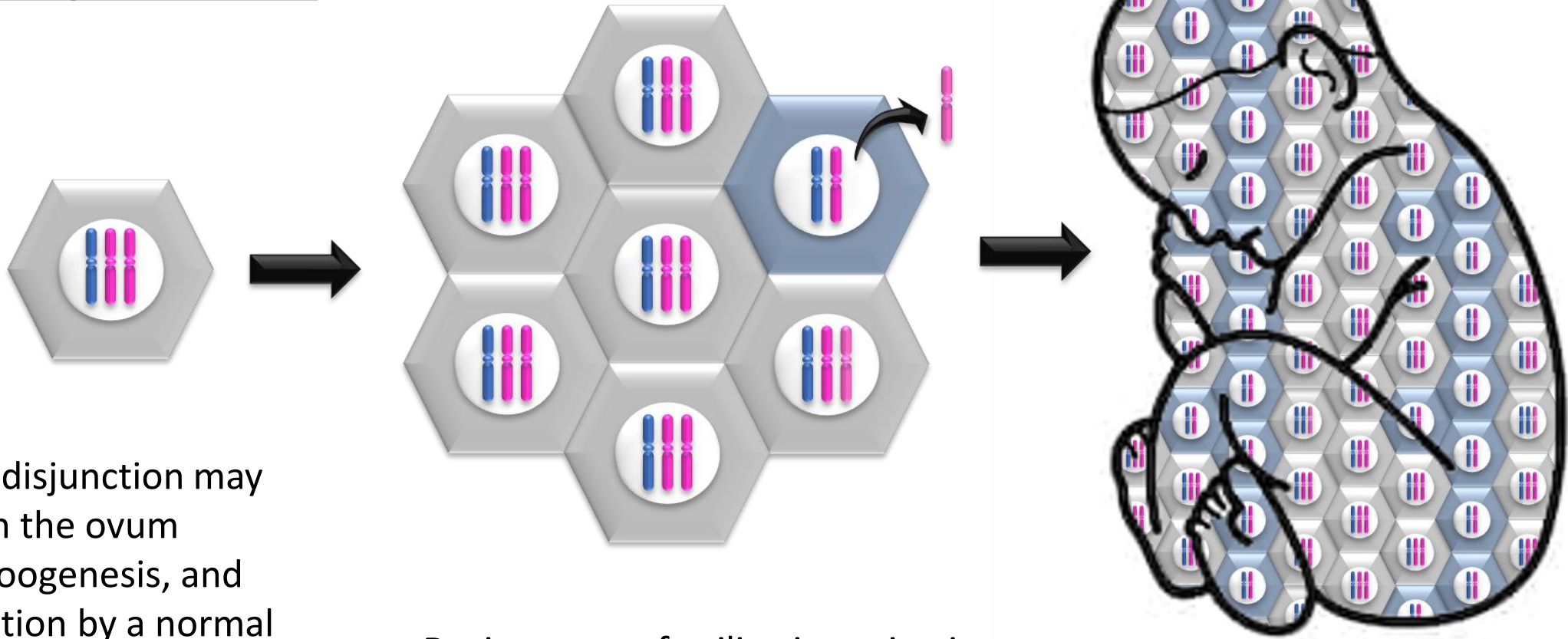
**This will lead to 2 cell types:**

- one continues as normal karyotype,
- the other with trisomy 21



# Consider 2 ways that a person could be mosaic for a Down syndrome:

## Trisomy Rescue

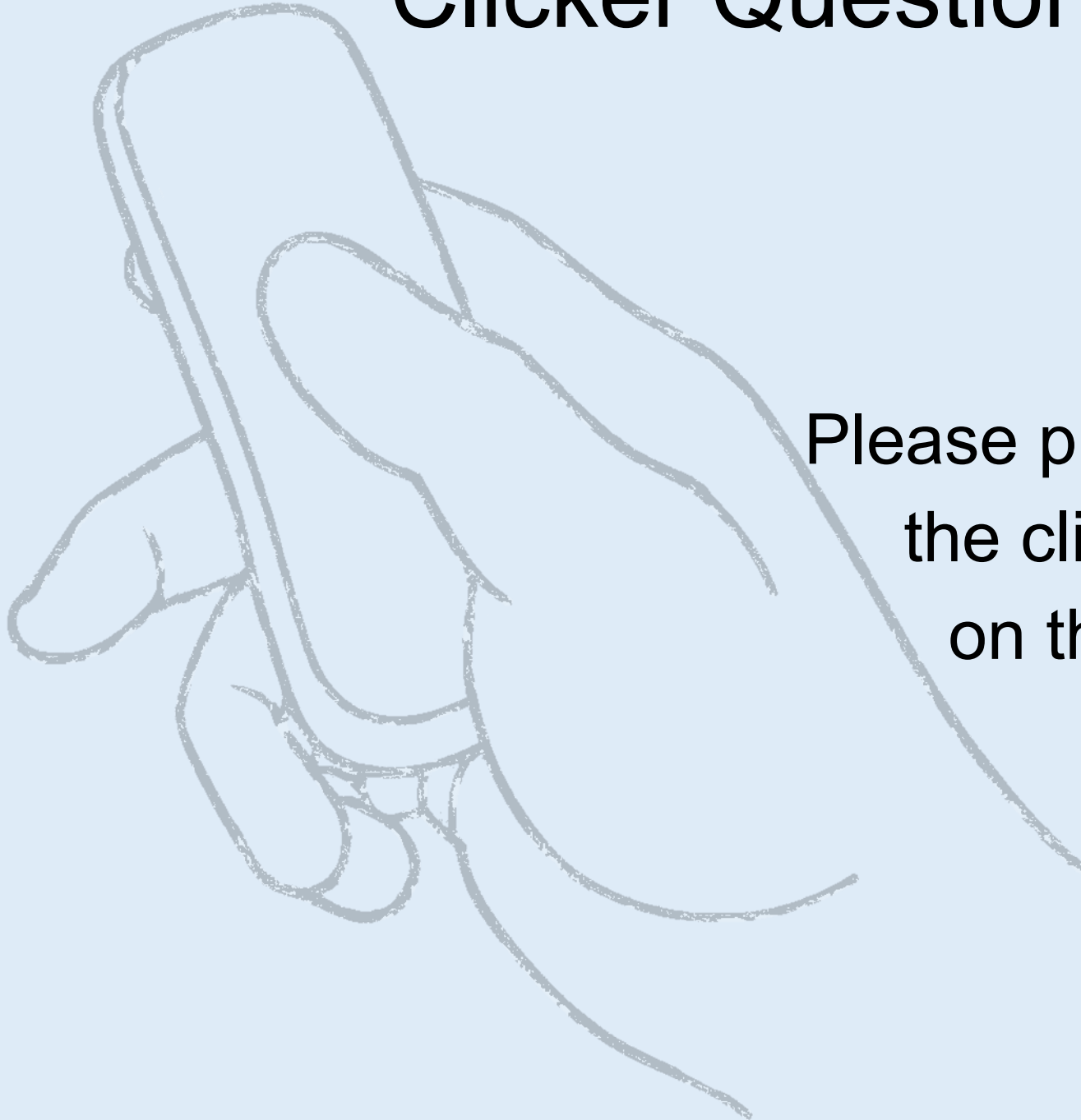


2. Non-disjunction may occur in the ovum during oogenesis, and fertilization by a normal sperm leads to trisomy 21 in the conception

During a post-fertilization mitosis, one of the extra chromosome 21 is evicted during cell division

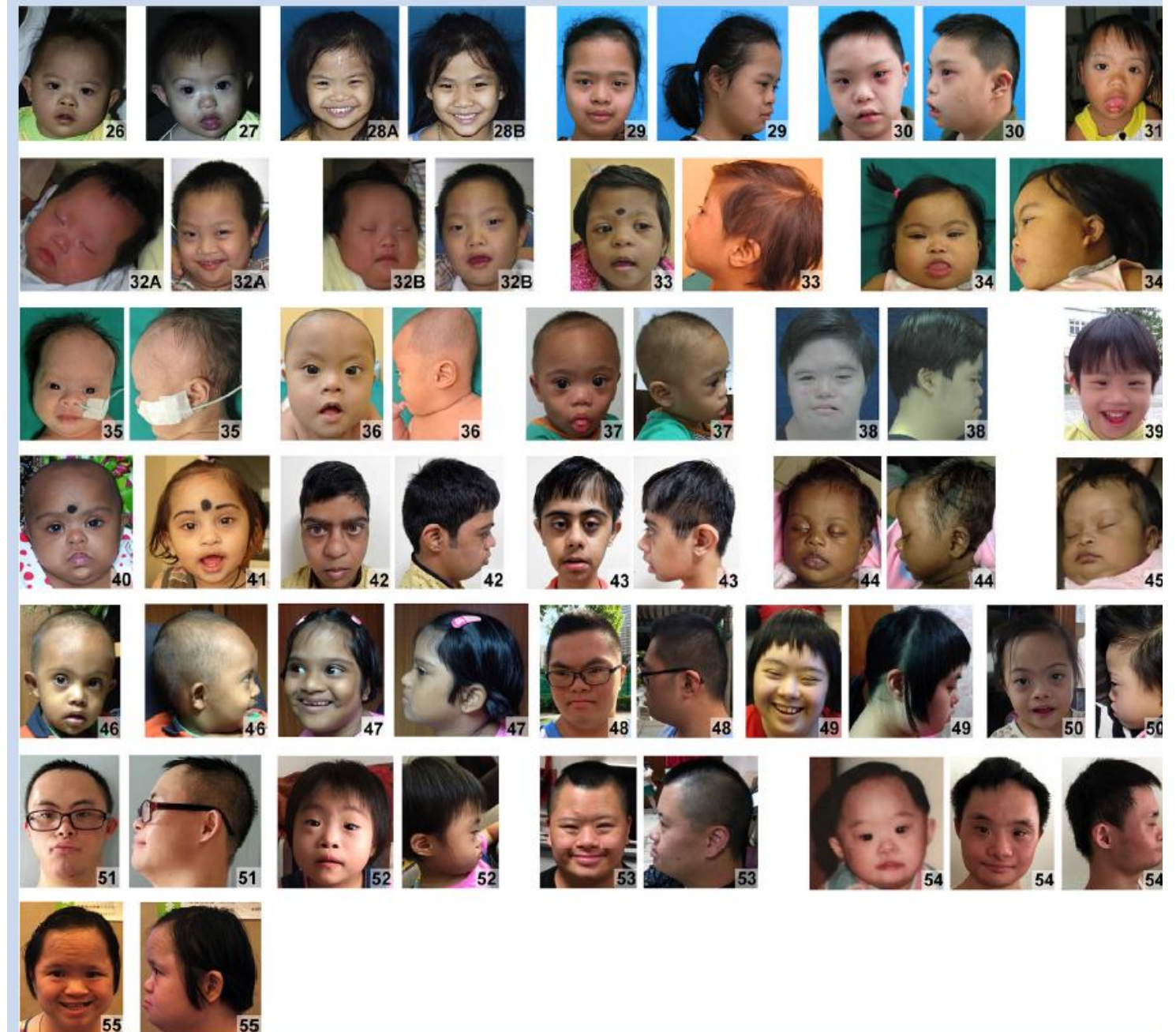
**Now the fetus has 2 cell types,** one with normal karyotype, and one that is trisomy 21

# Clicker Question Next!



Please prepare to answer  
the clicker question  
on the next slide

Down Syndrome in  
Diverse Populations  
Have a great day





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<https://www.youtube.com/watch?v=hA33eoPSLfo>